



User Guide

This report contains a wide range of useful information about the liver transplant program at University of Pittsburgh Medical Center. The report has three main sections:

- A. Program Summary
- B. Waiting List Information
- C. Transplant Information

The Program Summary is a one-page summary highlighting characteristics of the program, including the number of candidates on the waiting list, the number of transplants performed at the program, the number of patients being cared for by the program, and patient outcomes, including outcomes while on the waiting list (the transplant rate and the death rate while on the waiting list) and outcomes after transplant (patient and graft survival probabilities). If the program performed transplants in both adults and children, survival probabilities for adults and children (pediatrics) are provided separately. For each of the outcomes measures presented, a comparison is provided showing what would be expected at this program if it were performing as similar programs around the country perform when treating similar patients. More details regarding these outcome measures are provided in Sections B and C of the report.

The Waiting List Information section contains more detailed information on how many candidates are on the waiting list at the program, the types of candidates on the waiting list, how long candidates typically have to wait for a transplant at this program, how frequently candidates successfully receive a transplant, and how often candidates on the waiting list die before receiving a transplant.

Table B1 shows the activity on this program's waiting list during two recent 1-year periods and provides comparisons to all programs within this program's OPTN region (see <http://optn.transplant.hrsa.gov/members/regions.asp> for information on OPTN regions) and the nation as a whole. Tables B2 and B3 describe the candidates on the waiting list at this program, with comparisons to candidates waiting in the same donor service area (OPO/DSA) the OPTN region, and the nation as a whole.

Table B4 shows how many candidates were removed from the waiting list because they received a transplant. The program's transplant rate is calculated as the number of candidates who received a transplant divided by the person-years observed at the program (person-years is a combination of how many candidates were on the waiting list along with how long each candidate was followed since some candidates are not on the waiting list for the entire year). The transplant rate and comparisons to what would be expected at this program are presented in Figures B1 and B2. Figure B1 shows the transplant rate compared to what was expected at this program. The expected transplant rate is an estimate of what we would expect at this program if it were performing transplants at rates similar to other programs in the US with similar candidates on their waiting lists. The expected rate is only an estimate, and is made with a certain level of uncertainty. This uncertainty is shown in Figure B2. Figure B2 displays the ratio of the observed to the expected transplant rate. A ratio of 1 indicates that the observed transplant rate was equal to the expected transplant rate, while a ratio less than 1 indicates the observed rate was lower than expected rate and a ratio greater than 1 indicates the observed rate was higher than the expected rate. However, the level of uncertainty must be considered when interpreting these numbers. The 95% interval is also shown on Figure B2. This interval provides a range within which the true ratio of observed to expected transplant rates is likely to be. If this



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confidence interval includes (crosses) 1.0, then we cannot say that this program's observed transplant rate is different from what would be expected. The observed transplant rate at this program was 157.0 per 100 person-years. Transplant rates are also provided for adult and pediatric patients separately along with comparisons to adult and pediatric rates in the DSA, the OPTN region, and the nation. Transplant rates are also presented excluding transplants from a living donor (Table B4D and Figures B1D-B3D). Please refer to the PSR Technical Methods documentation available at <http://www.srtr.org> for more detail regarding how expected rates are calculated.

The pre-transplant mortality rate (previously called the waiting list mortality rate) for candidates on the waiting list is presented in Table B5 and Figures B4-B6. These data are presented in the same way as the transplant rate data in the previous section. The intent of this table and figures is to describe risk of death once candidates are listed rather than while they are listed, but before they are transplanted. Therefore, time at risk and deaths after removal from the waiting list for reasons other than transplant, transfer to another transplant program, or recovery (no longer needing a transplant), and before any subsequent transplant, are included. As with transplant rates, mortality rates should be interpreted carefully taking into consideration the interval displayed in Figure B5. For a complete description of how observed and expected mortality rates are calculated, please refer to the technical documentation available at <http://www.srtr.org>.

Survival from listing is presented in Table B6 and Figures B7-B9. These data are presented in the same way as the pre-transplant mortality rate data in the previous section. The intent of this table and figures is to describe risk of death once candidates are listed rather than while they are listed, including after a transplant. As with transplant rates, mortality rates should be interpreted carefully taking into consideration the interval displayed in Figure B8. For a complete description of how observed and expected mortality rates are calculated, please refer to the technical documentation available at <http://www.srtr.org>.

Table B7 presents information on what happens to candidates on the waiting list by three different time points after listing: 6 months, 12 months, and 18 months. The table displays percentages of candidates who have died, been removed from the waiting list, been transplanted, or been transferred or lost-to-follow-up. Tables B8 and B9 provide more detail regarding how many candidates have received a deceased donor transplant by certain time points during the first 3 years after being put on the transplant waiting list. Each row of Tables B8 and B9 presents the percent of candidates who received a deceased donor transplant by each time point. Table B10 presents data on the time it took for different percentages of patients to be transplanted for candidates added to the list between 01/01/2020 and 06/30/2025. The time it took for 5% (the 5th percentile) of patients to receive a transplant at this program was 0.1 months. If "Not Observed" is displayed in the table, then too few candidates received transplants before 12/31/2025 to calculate a particular percentile of transplant times.

Table B11 contains a summary of the offer acceptance practices of the program. The offer acceptance ratio indicates whether the program is more or less likely to accept offers than the average program. If the offer acceptance ratio is greater than 1.0, then the program tends to accept more offers than average; if the offer acceptance ratio is less than 1.0, then the program tends to accept fewer offers than average. Figure B10 shows the distribution of program offer acceptance rates as well as the offer acceptance rate for this program. Figures B11 - B15 similarly show offer acceptance rates for subsets



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of offers.

The Transplant Information section begins with descriptions of transplant recipients in Tables C1 and C2. Data on recipients of deceased donor transplants are presented (Tables C1D and C2D); if applicable, data on recipients of living donor transplants are presented separately (Tables C1L and C2L). Comparisons to the region and the nation as a whole are provided. A description of the deceased donors used at this program is provided in Table C3D, along with characteristics of living donors in Table C3L, if applicable. Finally, information on the transplant procedure for deceased and living donor transplants is presented in Tables C4D and C4L, respectively.

Starting with Table C5, transplant outcomes are presented along with comparisons to what would be expected at this program and what happened in the nation as a whole. Tables C5-C14 (tables C5-C10 for Pancreas) present information on graft survival (survival of the transplanted organ), with data presented separately for adult and pediatric recipients. Patients are followed from the time of transplant until either failure of the transplanted organ or death, whichever comes first. Please refer to the technical methods for more information on these calculations (<http://www.srtr.org>).

While Tables C5-C14 present data on graft survival, Tables C15-C20 (tables C11-C20 for Pancreas) present information on patient survival. For these tables, patients are followed from the time of transplant until death, regardless of whether the transplant is functioning or the patient required another transplant to survive.

Tables C21 and C22 summarize the multiorgan transplant outcomes at this program. The summary statistics in these tables are descriptive and are not risk-adjusted for different donor and candidate characteristics.

Table D1 shows the rates of follow-up for living donors.

Additional information regarding the technical methods and the risk adjustment models used to estimate expected event rates is available on the SRTR website at <http://www.srtr.org>. We welcome and encourage feedback on these reports. Please feel free to share feedback with the SRTR at the following e-mail: srtr@srtr.org.



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A. Program Summary

Figure A1. Waiting list and transplant activity

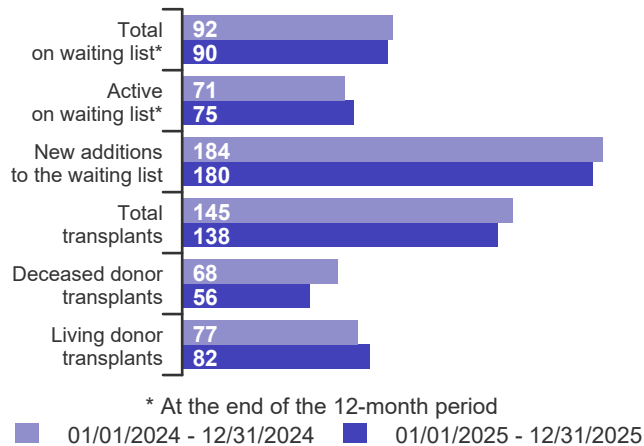


Table A1. Census of transplant recipients

Recipients	01/01/2024-12/31/2024	01/01/2025-12/31/2025
Transplanted at this center	145	138
Followed by this center*	1,283	1,319
...transplanted at this program	1,190	1,203
...transplanted elsewhere	93	116

* Recipients followed are transplant recipients for whom the center has submitted a post-transplant follow-up form for a transplant that took place before the 12-month interval for each column.

Figure A2. Transplant rates
01/01/2024 - 12/31/2025

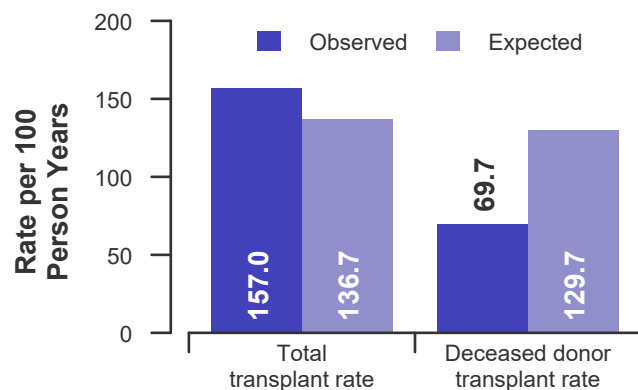


Figure A3. Pre-transplant mortality rates
01/01/2024 - 12/31/2025

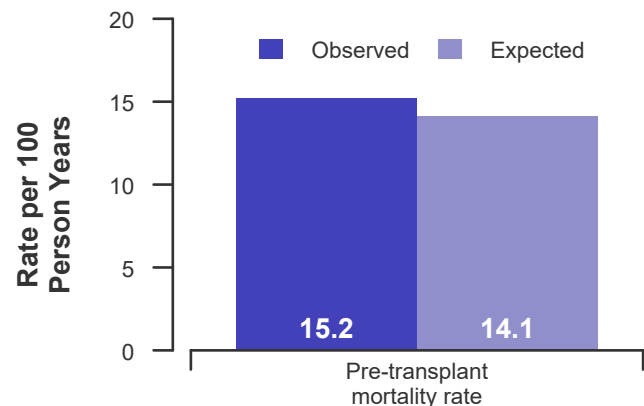


Figure A4. First-year adult graft and patient survival: 01/01/2023 - 06/30/2025

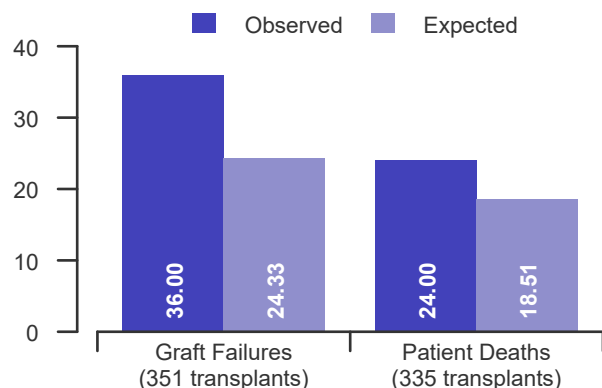


Figure A5. First-year pediatric graft and patient survival: 01/01/2023 - 06/30/2025

This center did not perform any transplants relevant to this figure during 01/01/2023-06/30/2025



B. Waiting List Information

Table B1. Waiting list activity summary: 01/01/2024 - 12/31/2025

Waiting List Registrations	Counts for this center		Activity for 01/01/2025 to 12/31/2025 as percent of registrants on waiting list on 01/01/2025		
	01/01/2024- 12/31/2024	01/01/2025- 12/31/2025	This Center (%)	OPTN Region (%)	U.S. (%)
On waiting list at start	93	92	100.0	100.0	100.0
Additions					
New listings at this center	184	180	195.7	117.1	169.4
Removals					
Transferred to another center	2	1	1.1	1.9	1.3
Received living donor transplant*	77	82	89.1	13.5	7.4
Received deceased donor transplant*	69	58	63.0	73.0	121.9
Died	11	7	7.6	9.7	8.6
Transplanted at another center	1	5	5.4	3.2	2.5
Deteriorated	12	12	13.0	7.2	9.1
Recovered	8	4	4.3	9.8	11.5
Other reasons	5	13	14.1	8.7	11.2
On waiting list at end of period	92	90	97.8	90.1	96.1

* These patients were removed from waiting list with removal code indicating transplant; this may not equal the number of transplants performed at this center during the specified period.



B. Waiting List Information

Table B2. Demographic characteristics of waiting list candidates**Candidates registered on the waiting list between 01/01/2025 and 12/31/2025**

Demographic Characteristic	New Waiting List Registrations 01/01/2025 to 12/31/2025 (%)			All Waiting List Registrations on 12/31/2025 (%)		
	This Center	OPTN Region	U.S.	This Center	OPTN Region	U.S.
	(N=180)	(N=1,503)	(N=16,177)	(N=90)	(N=1,156)	(N=9,175)
All (%)	100.0	100.0	100.0	100.0	100.0	100.0
Ethnicity/Race (%)*						
White	96.1	73.3	64.6	92.2	70.6	61.8
African-American	1.7	11.2	6.9	2.2	12.3	6.9
Hispanic/Latino	1.7	8.7	19.1	2.2	9.6	22.0
Asian	0.6	4.0	4.4	2.2	4.9	5.4
Other	0.0	0.3	2.2	0.0	0.4	2.3
Unknown	0.0	2.5	2.7	1.1	2.2	1.7
Age (%)						
<2 years	0.0	2.5	2.0	0.0	1.3	1.3
2-11 years	0.0	2.7	1.5	0.0	2.7	1.8
12-17 years	0.0	1.3	1.1	0.0	2.2	1.4
18-34 years	11.1	8.3	7.0	12.2	8.5	6.7
35-49 years	25.6	24.6	23.9	20.0	21.8	21.6
50-64 years	42.8	39.7	43.2	48.9	44.2	45.6
65-69 years	17.8	15.3	14.9	18.9	14.5	15.5
70+ years	2.8	5.7	6.5	0.0	4.8	6.0
Gender (%)						
Male	56.1	57.4	57.7	56.7	59.2	57.3
Female	43.9	42.6	42.3	43.3	40.8	42.7

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



B. Waiting List Information

Table B3. Medical characteristics of waiting list candidates

Candidates registered on the waiting list between 01/01/2025 and 12/31/2025

Medical Characteristic	New Waiting List Registrations 01/01/2025 to 12/31/2025 (%)			All Waiting List Registrations on 12/31/2025 (%)		
	This Center (N=180)	OPTN Region (N=1,503)	U.S. (N=16,177)	This Center (N=90)	OPTN Region (N=1,156)	U.S. (N=9,175)
All (%)	100.0	100.0	100.0	100.0	100.0	100.0
Blood Type (%)						
O	39.4	44.0	47.0	51.1	48.7	51.8
A	43.9	37.1	36.8	40.0	36.2	37.4
B	15.0	15.0	12.4	7.8	13.0	9.2
AB	1.7	3.8	3.8	1.1	2.1	1.6
Unknown	0.0	0.0	0.0	0.0	0.0	0.0
Previous Transplant (%)						
Yes	5.6	4.9	4.5	4.4	4.7	4.2
No	94.4	95.1	95.5	95.6	95.3	95.8
Unknown	0.0	0.0	0.0	0.0	0.0	0.0
Primary Disease (%)						
Acute Hepatic Necrosis	0.0	2.4	1.9	0.0	1.0	0.9
Non-Cholestatic Cirrhosis	7.8	7.9	9.9	23.3	27.2	22.6
Cholestatic Liver Disease/Cirrhosis	10.0	6.4	6.2	3.3	7.6	7.1
Biliary Atresia	0.6	1.8	1.6	1.1	2.1	2.0
Metabolic Diseases	2.2	3.0	1.8	3.3	2.7	1.4
Malignant Neoplasms	18.3	11.1	11.0	15.6	10.1	12.2
Other	61.1	67.1	67.5	53.3	49.0	53.5
Missing	0.0	0.4	0.1	0.0	0.3	0.1
Medical Urgency Status/MELD/PELD at Listing (%)*						
Status 1A	0.6	3.1	2.3	0.0	0.3	0.2
Status 1B	0.0	0.5	0.5	0.0	0.3	0.2
Status 2A	0.0	0.0	0.0	0.0	0.0	0.0
Status 2B	0.0	0.0	0.0	0.0	0.0	0.0
Status 3	0.0	0.0	0.0	0.0	0.0	0.0
MELD 6-10	12.8	12.2	11.1	14.4	26.1	20.3
MELD 11-14	16.1	11.6	12.0	20.0	20.2	21.5
MELD 15-20	23.3	19.8	22.6	36.7	28.1	31.2
MELD 21-30	21.7	20.8	23.1	24.4	15.4	14.0
MELD 31-40	9.4	17.4	13.6	2.2	1.6	1.2
PELD less than or equal to 10	0.0	2.5	1.2	0.0	2.4	1.9
PELD 11-14	0.0	0.3	0.3	0.0	0.3	0.2
PELD 15-20	0.0	0.5	0.4	0.0	0.2	0.3
PELD 21-30	0.0	0.3	0.2	0.0	0.1	0.1
PELD 31 or greater	0.0	0.0	0.1	0.0	0.0	0.0
Temporarily Inactive	15.0	6.5	8.5	2.2	4.9	8.8

* MELD/PELD score based on laboratory measures is shown for listings beginning 2/27/2002 unless patient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005.



B. Waiting List Information

Table B4. Transplant rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	93	182	1,371	10,135
Person Years**	182.2	365.8	2,547.6	19,006.5
Removals for Transplant	286	512	2,198	23,801
Adult (18+) Candidates				
Count on waiting list at start*	93	167	1,315	9,731
Person Years**	182.2	329.8	2,413.2	18,177.0
Removals for transplant	286	459	2,047	22,637
Pediatric (<18) Candidates				
Count on waiting list at start*	--	--	--	--
Person Years**	--	--	--	--
Removals for transplant	--	--	--	--

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, removal from the waiting list or December 31.

Figure B1. Observed and expected transplant rates: 01/01/2024 - 12/31/2025

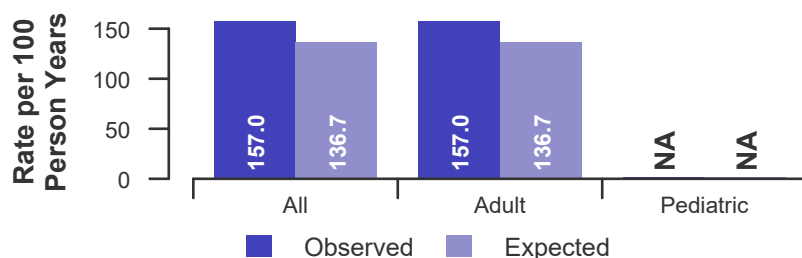


Figure B2. Transplant rate ratio estimate

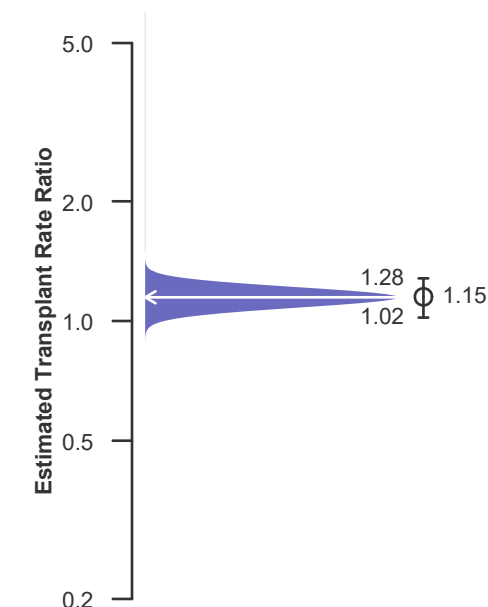
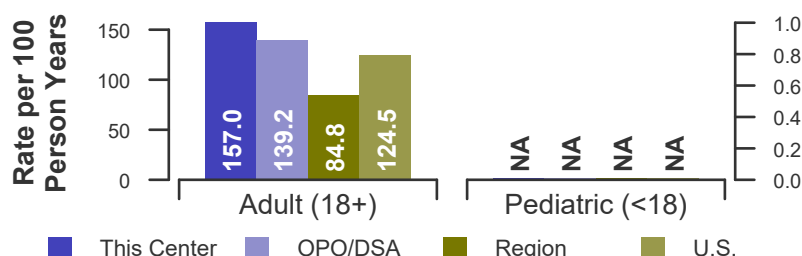


Figure B3. Observed adult (18+) and pediatric (<18) transplant rates: 01/01/2024 - 12/31/2025





B. Waiting List Information

Table B4D. Deceased donor transplant rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	93	182	1,371	10,135
Person Years**	182.2	365.8	2,547.6	19,006.5
Removals for Transplant	127	333	1,882	22,488
Adult (18+) Candidates				
Count on waiting list at start*	93	167	1,315	9,731
Person Years**	182.2	329.8	2,413.2	18,177.0
Removals for transplant	127	298	1,771	21,541
Pediatric (<18) Candidates				
Count on waiting list at start*	--	--	--	--
Person Years**	--	--	--	--
Removals for transplant	--	--	--	--

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, removal from the waiting list or December 31.

Figure B1D. Observed and expected deceased donor transplant rates: 01/01/2024 - 12/31/2025

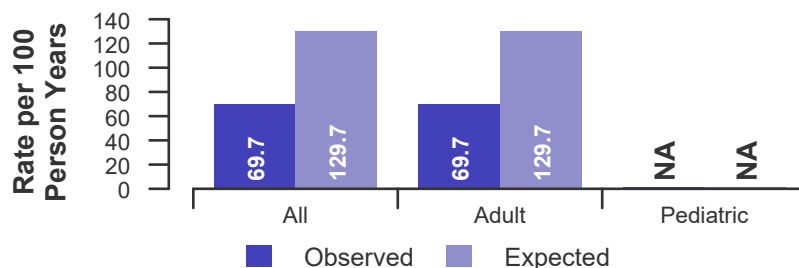


Figure B2D. Deceased donor transplant rate ratio estimate

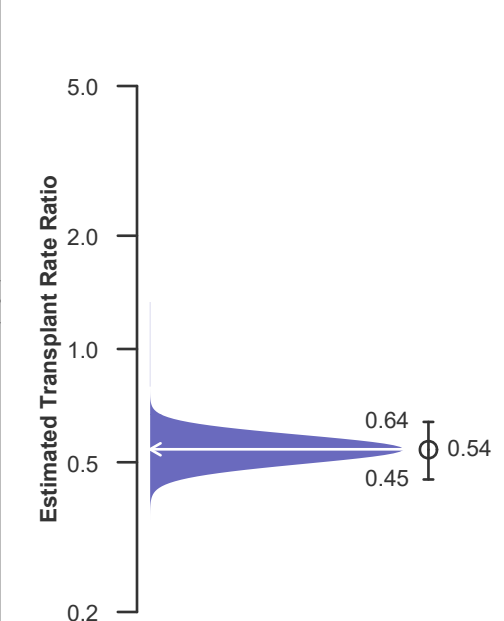
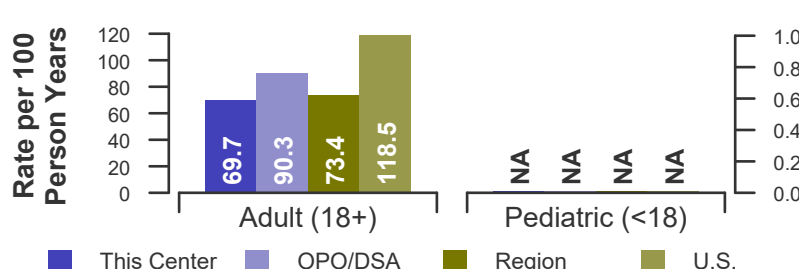


Figure B3D. Observed adult (18+) and pediatric (<18) deceased donor transplant rates: 01/01/2024 - 12/31/2025





B. Waiting List Information

Table B5. Pre-transplant mortality rates: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Candidates				
Count on waiting list at start*	93	182	1,371	10,135
Person Years**	210.6	420.3	2,939.2	22,423.8
Number of deaths	32	49	378	2,751
Adult (18+) Candidates				
Count on waiting list at start*	93	167	1,315	9,731
Person Years**	210.6	383.1	2,794.9	21,516.7
Number of deaths	32	48	371	2,718
Pediatric (<18) Candidates				
Count on waiting list at start*	--	--	--	--
Person Years**	--	--	--	--
Number of deaths	--	--	--	--

* Counts in this table may be lower than similar counts in other waiting list tables, such as Table B1. A small percentage (~1%) of patients are found to have died or been transplanted before being removed from the waiting list, so these patients are excluded if the event occurs prior to the start of the study period. Inactive time on the waiting list is included in the calculations for this table.

** Person years are calculated as days (converted to fractional years). The number of days from January 1 or from the date of first wait listing until death, transplant, 60 days after recovery, transfer or December 31.

Figure B4. Observed and expected pre-transplant mortality rates: 01/01/2024 - 12/31/2025

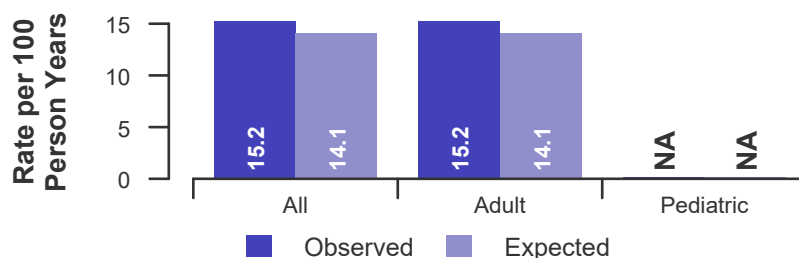


Figure B6. Observed adult (18+) and pediatric (<18) pre-transplant mortality rates: 01/01/2024 - 12/31/2025

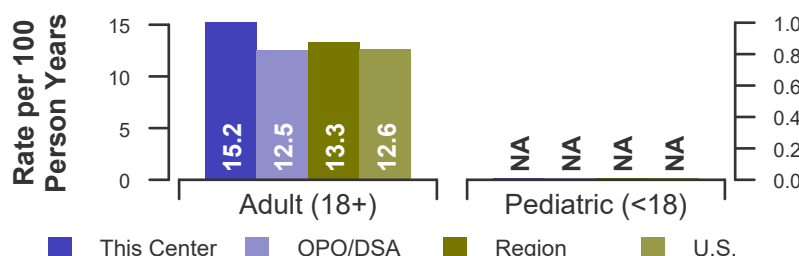
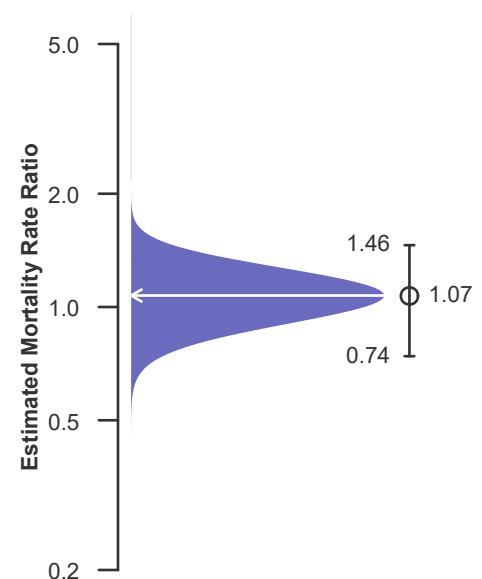


Figure B5. Pre-transplant mortality rate ratio estimate





B. Waiting List Information

Table B6. Rates of patient mortality after listing: 01/01/2024 - 12/31/2025

Waiting List Registrations	This Center	OPO/DSA	Region	U.S.
All Patients				
Count at risk during the evaluation period	1,071	1,926	8,986	85,845
Person-years*	1,430.3	2,579.4	12,096.4	115,619.4
Number of Deaths	91	143	753	6,194
Adult (18+) Patients				
Count at risk during the evaluation period	1,069	1,738	8,390	81,423
Person-years*	1,428.0	2,316.2	11,268.7	109,420.1
Number of Deaths	90	136	731	6,071
Pediatric (<18) Patients				
Count at risk during the evaluation period	2	188	596	4,422
Person-years*	2.3	263.2	827.7	6,199.3
Number of Deaths	1	7	22	123

* Person-years are calculated as days (converted to fractional years). The number of days from 01/01/2024, or from the date of first wait listing until death, reaching 5 years after listing or December 31, 2025.

** Patient mortality after listing describes the relative survival experience of patients after listing. It depends on many factors, some of which are outside of the control of the transplant program. For example, availability of organs may not be the same in every part of the country.

Figure B7. Observed and expected rates of patient mortality after listing: 01/01/2024 - 12/31/2025

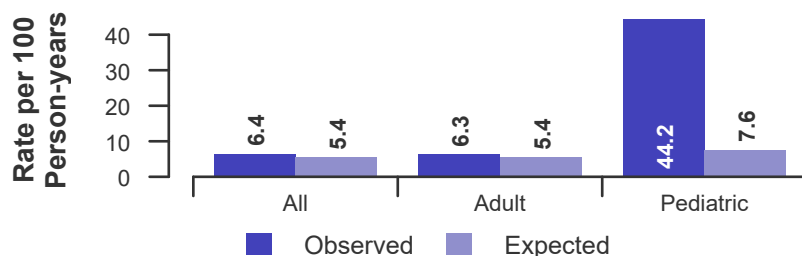


Figure B9. Observed adult (18+) and pediatric (<18) rates of patient mortality after listing: 01/01/2024 - 12/31/2025

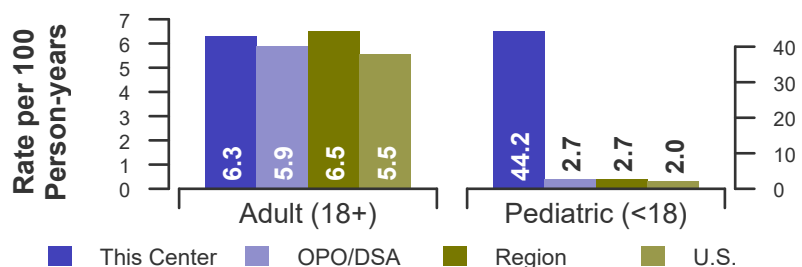
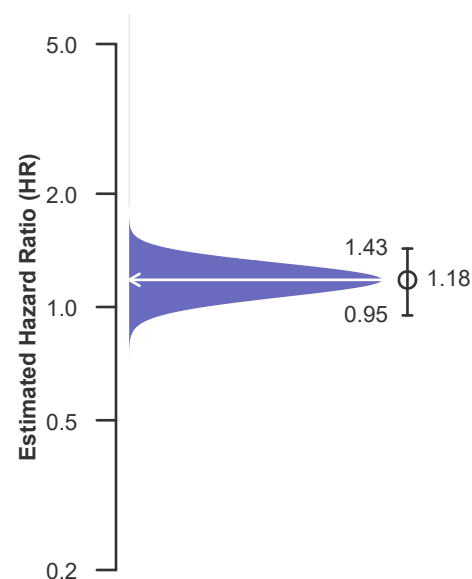


Figure B8. HR estimate of patient mortality after listing





B. Waiting List Information

Table B7. Waiting list candidate status after listing
Candidates registered on waiting list between 07/01/2023 and 06/30/2024

Waiting list status (survival status)	This Center (N=173)			U.S. (N=15,116)		
	Months Since Listing			Months Since Listing		
	6	12	18	6	12	18
Alive on waiting list (%)	34.1	16.2	9.2	32.7	17.0	10.7
Died on the waiting list without transplant (%)	2.9	4.6	4.6	3.7	4.5	4.9
Removed without transplant (%):						
Condition worsened (status unknown)	5.2	5.2	5.8	2.8	4.0	4.6
Condition improved (status unknown)	0.6	0.6	1.2	1.5	2.7	3.5
Refused transplant (status unknown)	0.0	0.0	0.0	0.2	0.3	0.5
Other	1.2	2.3	2.9	1.6	2.8	3.8
Transplant (living donor from waiting list only) (%):						
Functioning (alive)	32.4	35.3	22.5	2.5	3.1	2.0
Failed-Retransplanted (alive)	0.0	0.6	1.2	0.1	0.1	0.1
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.6	2.9	2.9	0.1	0.1	0.1
Status Yet Unknown**	0.6	1.7	15.6	0.0	0.1	1.4
Transplant (deceased donor) (%):						
Functioning (alive)	20.8	24.3	23.7	50.6	56.1	38.4
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.5	0.7	0.9
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	1.7	3.5	4.6	2.2	3.3	4.5
Status Yet Unknown*	0.0	1.7	4.6	1.1	4.4	23.8
Lost or Transferred (status unknown) (%)	0.0	1.2	1.2	0.3	0.6	0.7
TOTAL (%)	100.0	100.0	100.0	100.0	100.0	100.0
Total % known died on waiting list or after transplant	5.2	11.0	12.1	5.9	8.0	9.5
Total % known died or removed as unstable	10.4	16.2	17.9	8.8	12.0	14.2
Total % removed for transplant	56.1	69.9	75.1	57.2	68.0	71.3
Total % with known functioning transplant (alive)	53.2	59.5	46.2	53.2	59.2	40.5

* Follow-up form covering specified time period not yet completed, and possibly has not become due.



B. Waiting List Information

Table B7S1. Medical urgency status 1 candidate status after listing**Candidates registered on the waiting list between 07/01/2023 and 06/30/2024**

Waiting list status (survival status)	This Center (N=1)			U.S. (N=436)		
	Months Since listing			Months Since listing		
	6	12	18	6	12	18
Alive on waiting list (%)	0.0	0.0	0.0	0.7	0.2	0.0
Died on the waiting list without transplant (%)	0.0	0.0	0.0	6.7	6.7	6.7
Removed without transplant (%):						
Condition worsened (status unknown)	0.0	0.0	0.0	4.1	4.1	4.1
Condition improved (status unknown)	0.0	0.0	0.0	17.9	17.9	18.1
Refused transplant (status unknown)	0.0	0.0	0.0	0.0	0.0	0.0
Other	0.0	0.0	0.0	0.7	0.9	0.9
Transplant (living donor from waiting list only) (%):						
Functioning (alive)	0.0	0.0	0.0	1.4	1.4	0.2
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.2	0.2	0.2
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	0.0	0.0	0.0	0.2	0.2	0.2
Status Yet Unknown**	0.0	0.0	0.0	0.0	0.0	1.1
Transplant (deceased donor) (%):						
Functioning (alive)	0.0	0.0	0.0	58.3	52.3	34.9
Failed-Retransplanted (alive)	0.0	0.0	0.0	0.5	0.5	0.5
Failed-alive not retransplanted	0.0	0.0	0.0	0.0	0.0	0.0
Died	100.0	100.0	100.0	8.9	10.6	11.7
Status Yet Unknown*	0.0	0.0	0.0	0.5	4.8	21.1
Lost or Transferred (status unknown) (%)	0.0	0.0	0.0	0.0	0.2	0.2
TOTAL (%)	100.0	100.0	100.0	100.0	100.0	100.0
Total % known died on waiting list or after transplant	100.0	100.0	100.0	15.8	17.4	18.6
Total % known died or removed as unstable	100.0	100.0	100.0	20.0	21.6	22.7
Total % removed for transplant	100.0	100.0	100.0	70.0	70.0	70.0
Total % with known functioning transplant (alive)	0.0	0.0	0.0	59.6	53.7	35.1

* Follow-up form covering specified time period not yet completed, and possibly has not become due.



B. Waiting List Information

Table B8. Percent of candidates with deceased donor transplants: demographic characteristics
Candidates registered on the waiting list between 01/01/2020 and 12/31/2022

Characteristic	Percent transplanted at time periods since listing									
	This Center					United States				
	N	30 day	1 year	2 years	3 years	N	30 day	1 year	2 years	3 years
All	310	22.9	42.9	44.2	45.2	39,310	31.5	58.3	63.4	65.0
Ethnicity/Race*										
White	299	22.7	42.5	43.8	44.8	26,919	32.3	59.3	64.1	65.6
African-American	4	25.0	75.0	75.0	75.0	2,875	35.0	60.1	65.0	66.5
Hispanic/Latino	4	50.0	50.0	50.0	50.0	7,094	27.8	54.4	60.1	61.8
Asian	3	0.0	33.3	33.3	33.3	1,719	27.4	55.7	63.4	65.3
Other	0	--	--	--	--	701	33.2	56.5	61.9	63.8
Unknown	0	--	--	--	--	2	0.0	0.0	0.0	0.0
Age										
<2 years	0	--	--	--	--	782	26.0	71.0	74.0	75.1
2-11 years	0	--	--	--	--	588	29.4	67.3	71.4	73.8
12-17 years	0	--	--	--	--	501	27.9	62.1	67.5	68.5
18-34 years	17	35.3	47.1	47.1	47.1	2,627	43.6	62.5	66.7	67.9
35-49 years	72	34.7	51.4	51.4	51.4	8,163	43.6	63.5	67.3	68.5
50-64 years	142	23.9	42.3	43.0	45.1	18,162	29.8	57.5	63.0	64.8
65-69 years	57	5.3	35.1	36.8	36.8	6,238	21.1	52.6	58.9	60.5
70+ years	22	13.6	36.4	45.5	45.5	2,249	19.5	48.5	54.3	55.4
Gender										
Male	209	24.9	45.9	47.8	48.8	24,261	31.4	59.2	64.4	65.9
Female	101	18.8	36.6	36.6	37.6	15,049	31.6	56.7	61.7	63.4

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



B. Waiting List Information

Table B9. Percent of candidates with deceased donor transplants: medical characteristics
Candidates registered on the waiting list between 01/01/2020 and 12/31/2022

Characteristic	N	Percent transplanted at time periods since listing									
		This Center					United States				
		30 day	1 year	2 years	3 years		30 day	1 year	2 years	3 years	
All	310	22.9	42.9	44.2	45.2	39,310	31.5	58.3	63.4	65.0	
Blood Type											
O	127	19.7	36.2	37.8	39.4	18,395	30.2	56.1	61.5	63.1	
A	122	23.0	43.4	45.1	45.9	14,547	30.7	56.2	61.5	63.2	
B	45	28.9	53.3	53.3	53.3	4,794	34.5	66.3	70.7	72.0	
AB	16	31.2	62.5	62.5	62.5	1,574	45.2	77.9	80.2	80.7	
Previous Transplant											
Yes	14	50.0	57.1	57.1	57.1	1,794	36.0	58.2	62.4	64.4	
No	296	21.6	42.2	43.6	44.6	37,516	31.3	58.3	63.4	65.0	
Primary Disease											
Acute Hepatic Necrosis	9	22.2	22.2	22.2	22.2	1,914	59.5	67.8	68.6	68.8	
Non-Cholestatic Cirrhosis	164	18.9	39.6	40.2	41.5	24,341	32.8	57.7	62.3	63.8	
Cholestatic Liver Disease/Cirrhosis	19	15.8	21.1	21.1	26.3	2,417	25.9	56.1	63.6	66.1	
Biliary Atresia	0	--	--	--	--	632	18.4	70.1	74.8	76.6	
Metabolic Diseases	3	33.3	66.7	66.7	66.7	722	30.1	70.4	76.7	78.0	
Malignant Neoplasms	58	5.2	37.9	41.4	41.4	4,443	8.9	48.8	58.1	60.3	
Other	57	54.4	66.7	68.4	68.4	4,830	39.6	63.6	67.8	69.5	
Missing	0	--	--	--	--	11	45.5	54.5	63.6	63.6	
Medical Urgency Status/MELD/PELD at Listing*											
Status 1	0	--	--	--	--	0	--	--	--	--	
Status 1A	7	100.0	100.0	100.0	100.0	1,141	62.3	62.5	62.5	62.5	
Status 1B	0	--	--	--	--	149	50.3	79.2	79.2	79.2	
Status 2A	0	--	--	--	--	0	--	--	--	--	
Status 2B	0	--	--	--	--	0	--	--	--	--	
Status 3	0	--	--	--	--	0	--	--	--	--	
MELD 6-10	57	0.0	28.1	31.6	31.6	5,912	4.0	38.4	48.2	50.9	
MELD 11-14	42	4.8	21.4	23.8	23.8	4,528	4.9	37.0	46.9	50.0	
MELD 15-20	81	4.9	29.6	30.9	34.6	8,047	13.0	48.9	56.0	58.3	
MELD 21-30	75	34.7	58.7	58.7	58.7	9,720	39.9	69.6	72.1	72.8	
MELD 31-40	27	88.9	92.6	92.6	92.6	5,423	82.4	85.9	86.0	86.0	
PELD less than or equal to 10	0	--	--	--	--	565	13.6	69.9	76.3	79.1	
PELD 11-14	0	--	--	--	--	99	16.2	76.8	82.8	84.8	
PELD 15-20	0	--	--	--	--	129	21.7	82.9	83.7	85.3	
PELD 21-30	0	--	--	--	--	124	32.3	72.6	75.0	75.0	
PELD 31 or greater	0	--	--	--	--	57	54.4	71.9	71.9	71.9	
Temporarily Inactive	21	38.1	38.1	38.1	38.1	3,416	45.8	60.1	63.7	65.1	

* MELD/PELD score based on laboratory measures is shown for listings beginning 2/27/2002 unless patient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005.



B. Waiting List Information

Table B10. Time to transplant for waiting list candidates*

Candidates registered on the waiting list between 01/01/2020 and 06/30/2025

Percentile	Center	Months to Transplant**		U.S.
		OPO/DSA	Region	
5th	0.1	0.1	0.1	0.1
10th	0.2	0.2	0.2	0.2
25th	0.9	0.7	0.5	0.5
50th (median time to transplant)	3.6	3.8	6.1	4.0
75th	37.5	24.8	Not Observed	Not Observed

* If cells contain "Not Observed" fewer than that percentile of patients had received a transplant. For example, the 50th percentile of time to transplant is the time when 50% of candidates have received transplants. If waiting times are long, then the 50th percentile may not be observed during the follow-up period for this table. Also, if more than 50% of candidates are removed from the list due to death or other reasons before receiving transplants, then the 50th percentile of time to transplant will not be observed.

** Censored on 12/31/2025. Calculated as the months after listing, during which the corresponding percent of all patients initially listed had received a transplant.



B. Waiting List Information

Table B11. Offer Acceptance Practices: 01/01/2025 - 12/31/2025

Offers Acceptance Characteristics	This Center	OPO/DSA	Region	U.S.
Overall				
Number of Offers	1,837	4,570	34,682	235,608
Number of Acceptances	50	143	836	10,311
Expected Acceptances	90.0	240.9	1,229.8	10,310.0
Offer Acceptance Ratio*	0.56	0.60	0.68	1.00
95% Credible Interval**	[0.42, 0.73]	--	--	--
PHS increased infectious risk				
Number of Offers	179	433	3,577	21,582
Number of Acceptances	9	18	120	1,417
Expected Acceptances	12.9	26.5	161.7	1,414.2
Offer Acceptance Ratio*	0.74	0.70	0.75	1.00
95% Credible Interval**	[0.37, 1.23]	--	--	--
DCD donor				
Number of Offers	1,259	2,102	19,214	143,677
Number of Acceptances	23	59	281	4,151
Expected Acceptances	46.7	112.1	454.1	4,152.2
Offer Acceptance Ratio*	0.51	0.53	0.62	1.00
95% Credible Interval**	[0.33, 0.73]	--	--	--
HCV+ donor				
Number of Offers	59	90	774	6,087
Number of Acceptances	1	1	26	269
Expected Acceptances	3.1	4.4	36.5	269.6
Offer Acceptance Ratio*	0.58	0.47	0.73	1.00
95% Credible Interval**	[0.12, 1.40]	--	--	--
Hard-to-Place Livers (Over 50 Offers)				
Number of Offers	757	1,402	16,533	112,282
Number of Acceptances	4	8	71	1,037
Expected Acceptances	6.8	28.7	188.9	1,220.8
Offer Acceptance Ratio*	0.69	0.33	0.38	0.85
95% Credible Interval**	[0.25, 1.33]	--	--	--
Donor more than 500 miles away				
Number of Offers	148	562	3,186	53,033
Number of Acceptances	6	12	64	1,220
Expected Acceptances	6.5	17.6	90.7	1,091.7
Offer Acceptance Ratio*	0.95	0.71	0.71	1.12
95% Credible Interval**	[0.41, 1.71]	--	--	--

* The offer acceptance ratio estimates the relative offer acceptance practice of University of Pittsburgh Medical Center compared to the national offer acceptance practice. A ratio above one indicates the program accepts more offers compared to national offer acceptance practices (e.g., an offer acceptance ratio of 1.25 indicates a center accepts 25% more offers than is expected based on national offer acceptance practices), while a ratio below one indicates the program accepts fewer offers compared to national offer acceptance practices (e.g., an offer acceptance ratio of 0.75 indicates a center accepts 25% fewer offers than is expected based on national offer acceptance practices).

** As an example, the 95% Credible Interval for the overall offer acceptance ratio, [0.42, 0.73], indicates the location of PAPT's true offer acceptance ratio with 95% probability. The best estimate is 44% less likely to accept an offer compared to national acceptance behavior, but PAPT's performance could plausibly range from 58% reduced acceptance up to 27% reduced acceptance.



B. Waiting List Information

Figure B10. Offer acceptance: Overall

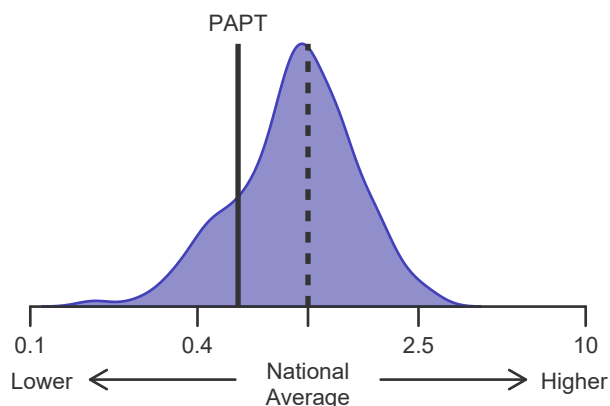


Figure B11. Offer acceptance: PHS increased infectious risk

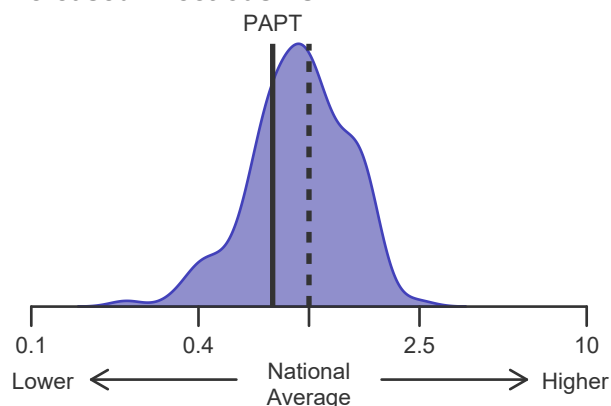


Figure B12. Offer acceptance: DCD Donor

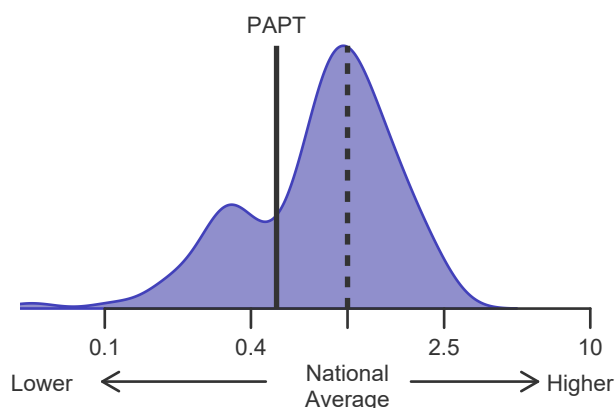


Figure B13. Offer acceptance: HCV+ Donor

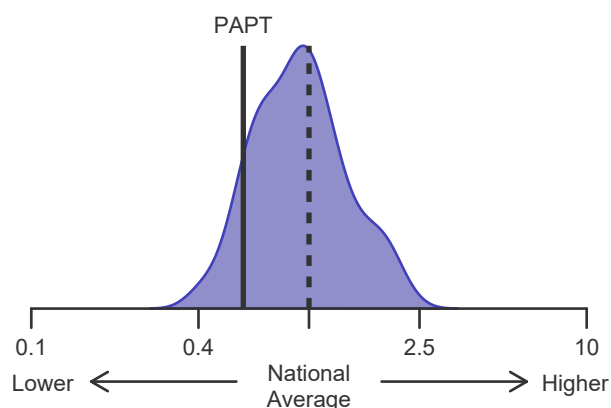


Figure B14. Offer acceptance: Offer number > 50

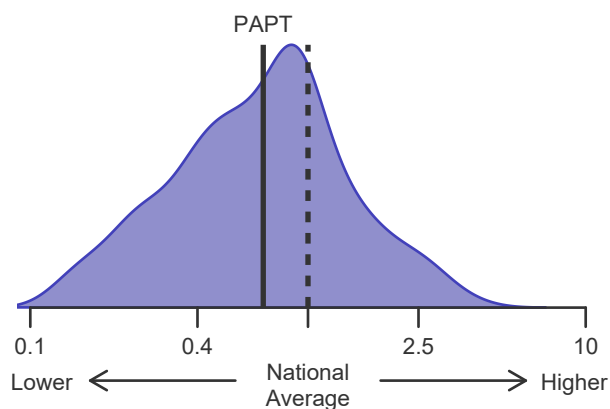
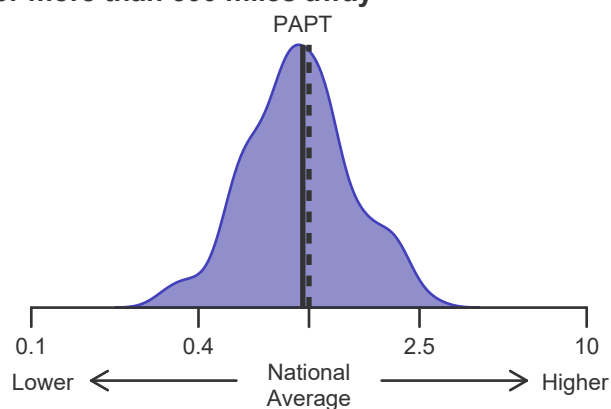


Figure B15. Offer acceptance: Donor more than 500 miles away





C. Transplant Information

Table C1D. Deceased donor transplant recipient demographic characteristics

Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=56)	Region (N=897)	U.S. (N=10,771)
Ethnicity/Race (%)*			
White	98.2	71.6	66.3
African-American	0.0	11.9	6.8
Hispanic/Latino	1.8	9.6	18.0
Asian	0.0	3.8	4.1
Other	0.0	0.7	2.1
Unknown	0.0	2.5	2.7
Age (%)			
<2 years	0.0	1.7	1.6
2-11 years	0.0	2.8	1.5
12-17	0.0	1.2	1.1
18-34	3.6	6.1	7.1
35-49 years	25.0	24.4	24.0
50-64 years	57.1	42.4	42.8
65-69 years	10.7	15.7	15.1
70+ years	3.6	5.7	6.9
Gender (%)			
Male	60.7	60.3	59.2
Female	39.3	39.7	40.8

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C1L. Living donor transplant recipient demographic characteristics

Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=82)	Region (N=173)	U.S. (N=707)
Ethnicity/Race (%)*			
White	97.6	89.0	74.5
African-American	1.2	5.2	4.1
Hispanic/Latino	1.2	2.9	15.1
Asian	0.0	2.3	3.5
Other	0.0	0.0	0.6
Unknown	0.0	0.6	2.1
Age (%)			
<2 years	0.0	6.4	9.9
2-11 years	0.0	4.0	3.7
12-17	0.0	0.0	1.3
18-34	12.2	8.7	8.6
35-49 years	28.0	25.4	21.8
50-64 years	34.1	34.1	36.2
65-69 years	15.9	13.9	12.4
70+ years	9.8	7.5	6.1
Gender (%)			
Male	56.1	50.9	49.5
Female	43.9	49.1	50.5

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C2D. Deceased donor transplant recipient medical characteristics
Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=56)	Region (N=897)	U.S. (N=10,771)
Blood Type (%)			
O	33.9	43.5	45.7
A	48.2	37.6	36.3
B	14.3	15.3	13.5
AB	3.6	3.7	4.5
Previous Transplant (%)			
Yes	10.7	4.8	4.2
No	89.3	95.2	95.8
Body Mass Index (%)			
0-20	1.8	11.5	9.4
21-25	21.4	26.4	25.9
26-30	35.7	30.5	30.5
31-35	17.9	17.7	19.3
36-40	12.5	8.4	8.6
41+	7.1	3.9	4.3
Unknown	3.6	1.6	1.9
Primary Disease (%)			
Acute Hepatic Necrosis	0.0	2.6	2.0
Non-Cholestatic Cirrhosis	5.4	7.0	10.2
Cholestatic Liver Disease/Cirrhosis	1.8	4.8	6.2
Biliary Atresia	0.0	1.2	1.5
Metabolic Diseases	0.0	3.7	2.1
Malignant Neoplasms	14.3	16.5	12.5
Other	78.6	64.1	65.5
Missing	0.0	0.1	0.1
Medical Urgency Statust/MELD/PELD at Transplant (%)*			
Status 1A	1.8	3.5	2.6
Status 1B	0.0	3.0	1.2
MELD 6-10	8.9	11.6	10.2
MELD 11-14	7.1	6.5	9.2
MELD 15-20	16.1	14.9	19.7
MELD 21-30	32.1	25.3	29.0
MELD 31-40	30.4	25.1	20.1
PELD less than or equal to 10	0.0	1.1	0.8
PELD 11-14	0.0	0.2	0.3
PELD 15-20	0.0	0.0	0.2
PELD 21-30	0.0	0.3	0.3
PELD 31 or greater	0.0	0.0	0.1
Temporarily Inactive	0.0	0.0	0.0
Recipient Medical Condition at Transplant (%)			
Not Hospitalized	66.1	51.6	59.7
Hospitalized	25.0	34.8	24.7
ICU	8.9	13.5	15.6
Unknown	0.0	0.1	0.0

* MELD/PELD score based on laboratory measures at the time of transplant is shown unless recipient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005



C. Transplant Information

Table C2L. Living donor transplant recipient medical characteristics
Patients transplanted between 01/01/2025 and 12/31/2025

Characteristic	Percentage in each category		
	Center (N=82)	Region (N=173)	U.S. (N=707)
Blood Type (%)			
O	36.6	41.6	44.4
A	47.6	47.4	44.1
B	14.6	9.2	9.3
AB	1.2	1.7	2.1
Previous Transplant (%)			
Yes	1.2	1.2	1.8
No	98.8	98.8	98.2
Body Mass Index (%)			
0-20	9.8	16.2	21.5
21-25	28.0	34.7	31.5
26-30	31.7	27.2	27.6
31-35	15.9	13.3	12.4
36-40	8.5	5.8	4.5
41+	4.9	2.3	1.7
Unknown	1.2	0.6	0.7
Primary Disease (%)			
Acute Hepatic Necrosis	0.0	0.0	0.4
Non-Cholestatic Cirrhosis	6.1	6.4	9.2
Cholestatic Liver Disease/Cirrhosis	19.5	23.7	18.0
Biliary Atresia	1.2	6.4	10.3
Metabolic Diseases	0.0	2.3	2.3
Malignant Neoplasms	20.7	17.3	12.4
Other	52.4	43.9	47.4
Missing	0.0	0.0	0.0
Medical Urgency Statust/MELD/PELD at Transplant (%)*			
Status 1A	0.0	0.0	0.7
Status 1B	0.0	1.2	1.7
MELD 6-10	22.0	25.4	23.8
MELD 11-14	25.6	23.1	21.8
MELD 15-20	29.3	26.0	24.3
MELD 21-30	13.4	10.4	11.2
MELD 31-40	1.2	0.6	0.8
PELD less than or equal to 10	0.0	4.6	4.8
PELD 11-14	0.0	1.2	1.6
PELD 15-20	0.0	0.6	1.7
PELD 21-30	0.0	1.7	2.1
PELD 31 or greater	0.0	0.6	0.8
Temporarily Inactive	8.5	4.6	4.7
Recipient Medical Condition at Transplant (%)			
Not Hospitalized	96.3	90.2	87.6
Hospitalized	3.7	8.7	10.2
ICU	0.0	1.2	2.1
Unknown	0.0	0.0	0.1

* MELD/PELD score based on laboratory measures at the time of transplant is shown unless recipient is Status 1 or Temporarily Inactive. MELD/PELD scores based on exception rules are not used. Status 1 separated into 1A and 1B in August 2005



C. Transplant Information

Table C3D. Deceased donor characteristics

Transplants performed between 01/01/2025 and 12/31/2025

Donor Characteristic	Percentage in each category		
	Center (N=56)	Region (N=897)	U.S. (N=10,771)
Cause of Death (%)			
Deceased: Stroke	21.4	27.9	29.4
Deceased: MVA	12.5	9.6	9.6
Deceased: Other	66.1	62.5	61.0
Ethnicity/Race (%)*			
White	73.2	64.3	63.7
African-American	19.6	22.7	17.1
Hispanic/Latino	3.6	8.7	13.9
Asian	3.6	2.3	3.1
Other	0.0	0.2	1.2
Not Reported	0.0	1.7	1.0
Age (%)			
<2 years	0.0	0.7	0.6
2-11 years	0.0	2.0	1.7
12-17	1.8	3.8	2.9
18-34	28.6	17.3	18.0
35-49 years	32.1	28.5	27.6
50-64 years	33.9	32.1	32.2
65-69 years	3.6	9.5	9.7
70+ years	0.0	6.1	7.3
Gender (%)			
Male	58.9	59.8	61.0
Female	41.1	40.2	39.0
Blood Type (%)			
O	39.3	48.4	48.9
A	51.8	38.0	36.7
B	7.1	12.0	11.7
AB	1.8	1.6	2.7
Unknown	0.0	0.0	0.0

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C3L. Living donor characteristics

Transplants performed between 01/01/2025 and 12/31/2025

Donor Characteristic	Percentage in each category		
	Center (N=82)	Region (N=173)	U.S. (N=707)
Ethnicity/Race (%)*			
White	62.2	73.4	72.6
African-American	1.2	5.2	3.3
Hispanic/Latino	0.0	0.6	13.0
Asian	0.0	0.0	3.1
Other	0.0	0.6	1.1
Not Reported	36.6	20.2	6.9
Age (%)			
0-11 years	0.0	0.6	0.6
12-17	0.0	0.0	0.1
18-34	28.0	34.7	37.2
35-49 years	52.4	48.0	46.0
50-64 years	19.5	16.2	16.0
65-69 years	0.0	0.0	0.0
70+ years	0.0	0.6	0.1
Gender (%)			
Male	36.6	41.6	41.6
Female	63.4	58.4	58.4
Blood Type (%)			
O	51.2	59.5	62.0
A	39.0	35.3	32.5
B	9.8	5.2	5.0
AB	0.0	0.0	0.6
Unknown	0.0	0.0	0.0

* Race and ethnicity are reported together as a single data element, reflecting their data collection (either race or ethnicity is required, but not both). Patients formerly coded as white and Hispanic are coded as Hispanic. Race and ethnicity sum to 100%.



C. Transplant Information

Table C4D. Deceased donor transplant characteristics
Transplants performed between 01/01/2025 and 12/31/2025

Transplant Characteristic	Percentage in each category		
	Center (N=56)	Region (N=897)	U.S. (N=10,771)
Cold Ischemic Time (Hours): Local (%)			
Deceased: 0-5 hr	38.9	46.1	32.5
Deceased: 6-10 hr	33.3	18.6	16.9
Deceased: 11-15 hr	16.7	15.9	17.3
Deceased: 16-20 hr	0.0	13.6	20.6
Deceased: 21+ hr	11.1	5.1	12.1
Not Reported	0.0	0.7	0.6
Cold Ischemic Time (Hours): Shared (%)			
Deceased: 0-5 hr	13.2	19.6	18.2
Deceased: 6-10 hr	55.3	41.2	28.8
Deceased: 11-15 hr	15.8	17.1	17.6
Deceased: 16-20 hr	15.8	13.5	20.9
Deceased: 21+ hr	0.0	8.5	14.2
Not Reported	0.0	0.2	0.4
Procedure Type (%)			
Single organ	89.3	97.1	98.3
Multi organ	10.7	2.9	1.7
Donor Location (%)			
Local Donation Service Area (DSA)	32.1	32.9	36.8
Another Donation Service Area (DSA)	67.9	67.1	63.2
Median Time in Hospital After Transplant	10.0 Days	11.5 Days	10.0 Days



C. Transplant Information

Table C4L. Living donor transplant characteristics
Transplants performed between 01/01/2025 and 12/31/2025

Transplant Characteristic	Percentage in each category		
	Center (N=82)	Region (N=173)	U.S. (N=707)
Relation with Donor (%)			
Related	42.7	46.8	48.4
Unrelated	57.3	52.0	51.2
Not Reported	0.0	1.2	0.4
Procedure Type (%)			
Single organ	98.8	99.4	99.9
Multi organ	1.2	0.6	0.1
Median Time in Hospital After Transplant	7.0 Days	9.0 Days	10.0 Days



C. Transplant Information

Table C5. Adult (18+) 1-month survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	351	24,637
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	97.44% [95.80%-99.10%]	97.29% [97.09%-97.50%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	96.97%	--
Number of observed graft failures (including deaths) during the first month after transplant	9	667
Number of expected graft failures (including deaths) during the first month after transplant	11.17	--
Estimated hazard ratio*	0.84	--
95% credible interval for the hazard ratio**	[0.42, 1.40]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.42, 1.40], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 16% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 58% reduced risk up to 40% increased risk.

Figure C1. Adult (18+) 1-month graft failure HR estimate

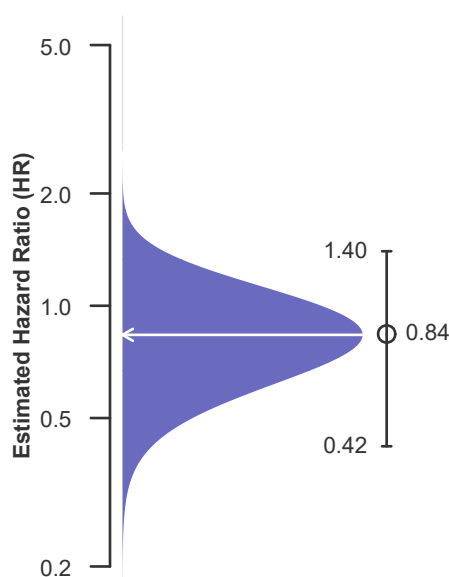
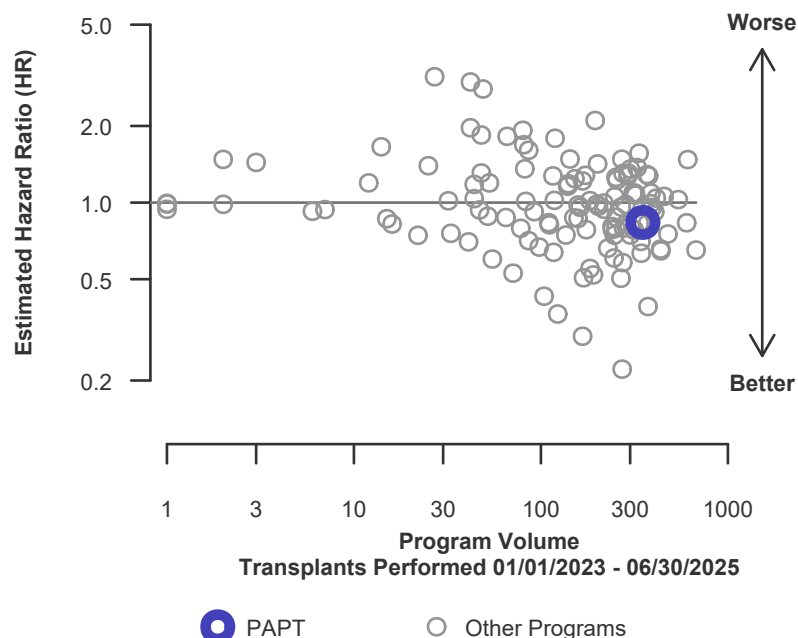


Figure C2. Adult (18+) 1-month graft failure HR program comparison





C. Transplant Information

Table C5D. Adult (18+) 1-month survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	150	23,288
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	97.33% [94.79%-99.95%]	97.31% [97.10%-97.52%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	97.25%	--
Number of observed graft failures (including deaths) during the first month after transplant	4	626
Number of expected graft failures (including deaths) during the first month after transplant	4.18	--
Estimated hazard ratio*	0.97	--
95% credible interval for the hazard ratio**	[0.36, 1.89]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.36, 1.89], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 3% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 64% reduced risk up to 89% increased risk.

Figure C1D. Adult (18+) 1-month deceased donor graft failure HR estimate

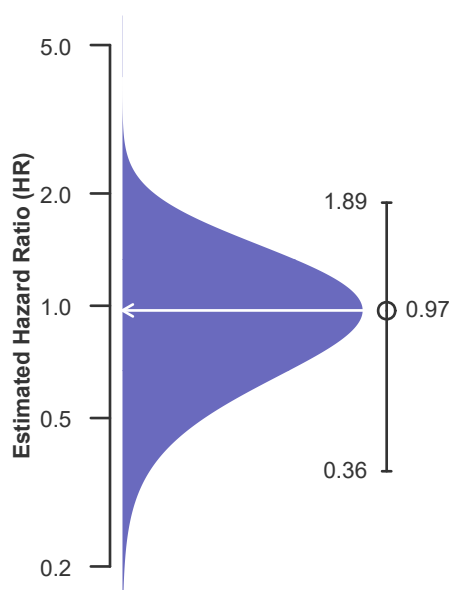
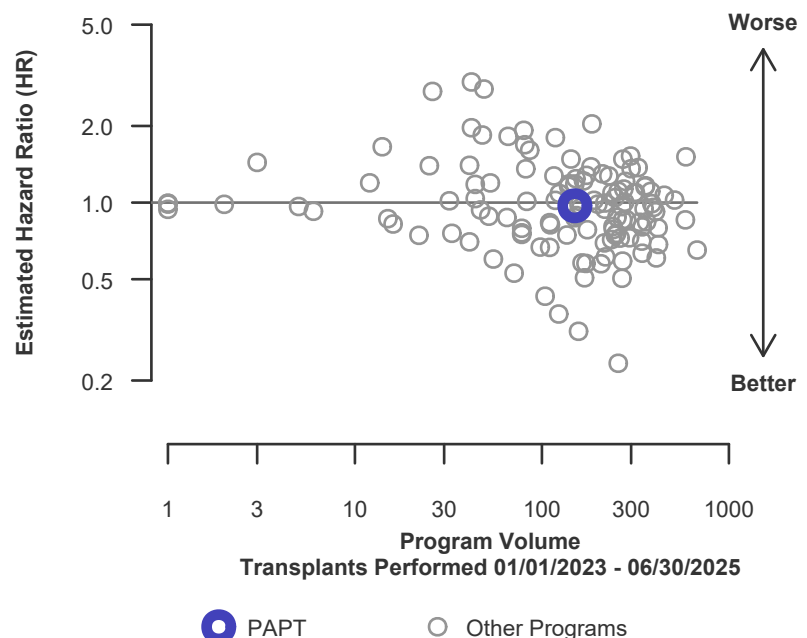


Figure C2D. Adult (18+) 1-month deceased donor graft failure HR program comparison





C. Transplant Information

Table C5L. Adult (18+) 1-month survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	201	1,349
Estimated probability of surviving with a functioning graft at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	97.51% [95.38%-99.69%]	96.96% [96.05%-97.88%]
Expected probability of surviving with a functioning graft at 1 month (adjusted for patient and donor characteristics)	96.76%	--
Number of observed graft failures (including deaths) during the first month after transplant	5	41
Number of expected graft failures (including deaths) during the first month after transplant	6.99	--
Estimated hazard ratio*	0.78	--
95% credible interval for the hazard ratio**	[0.31, 1.45]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.31, 1.45], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 22% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 69% reduced risk up to 45% increased risk.

Figure C1L. Adult (18+) 1-month living donor graft failure HR estimate

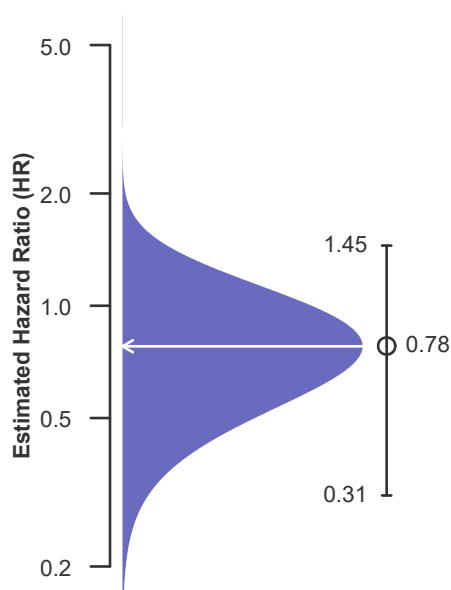
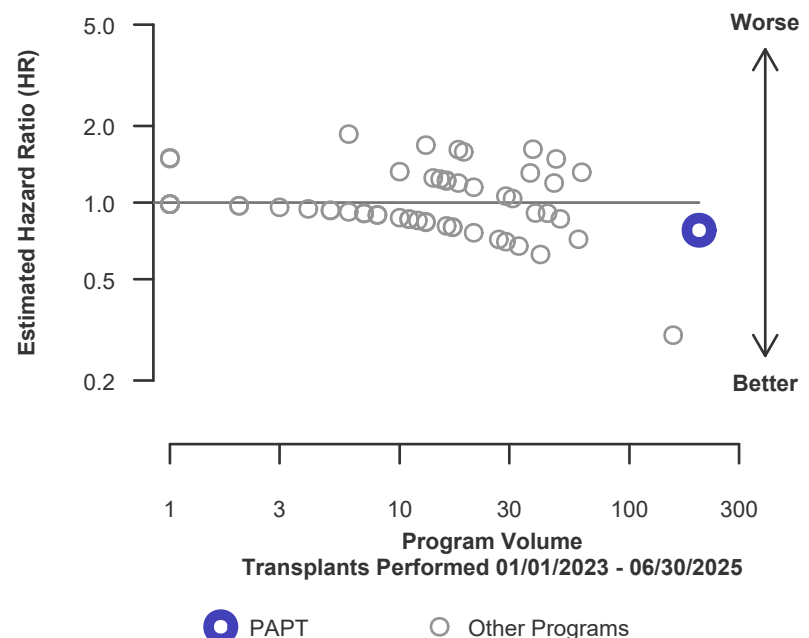


Figure C2L. Adult (18+) 1-month living donor graft failure HR program comparison





C. Transplant Information

Table C6. Adult (18+) 90-Day survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	351	24,637
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	94.30% [91.91%-96.76%]	95.59% [95.34%-95.85%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.53%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	20	1,086
Number of expected graft failures (including deaths) during the first 90 days after transplant	16.22	--
Estimated hazard ratio*	1.21	--
95% credible interval for the hazard ratio**	[0.76, 1.76]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.76, 1.76], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 21% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 24% reduced risk up to 76% increased risk.

Figure C3. Adult (18+) 90-Day graft failure HR estimate

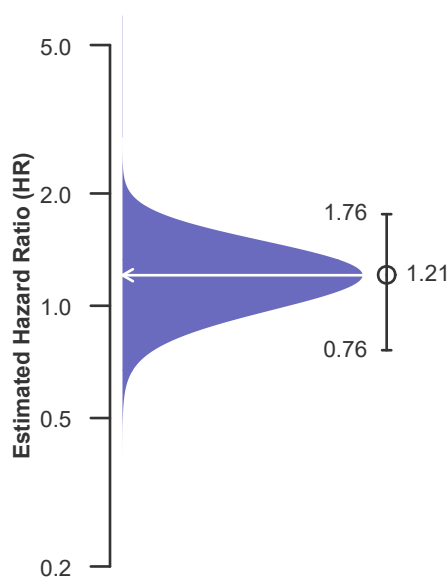
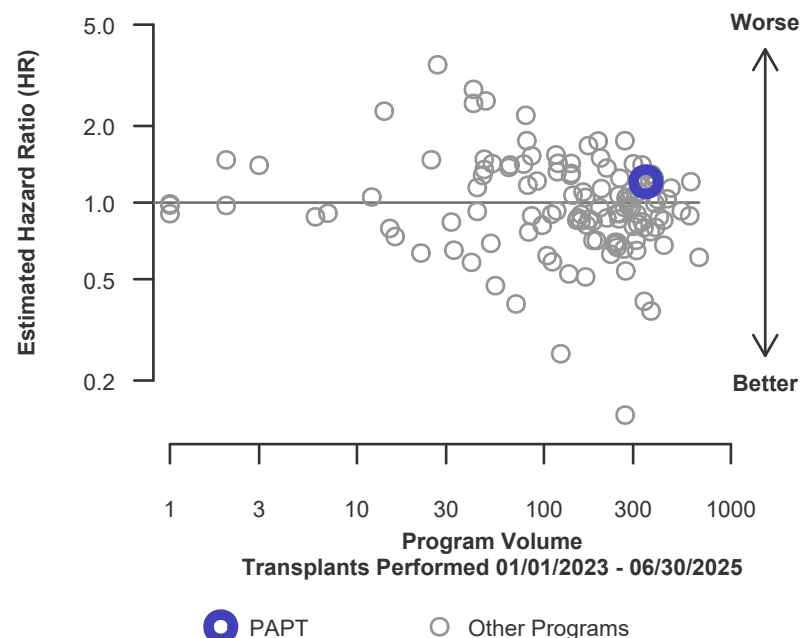


Figure C4. Adult (18+) 90-Day graft failure HR program comparison





C. Transplant Information

Table C6D. Adult (18+) 90-Day survival with a functioning deceased donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	150	23,288
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	92.67% [88.59%-96.93%]	95.58% [95.31%-95.84%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.43%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	11	1,030
Number of expected graft failures (including deaths) during the first 90 days after transplant	6.82	--
Estimated hazard ratio*	1.47	--
95% credible interval for the hazard ratio**	[0.78, 2.38]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.78, 2.38], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 47% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 22% reduced risk up to 138% increased risk.

Figure C3D. Adult (18+) 90-Day deceased donor graft failure HR estimate

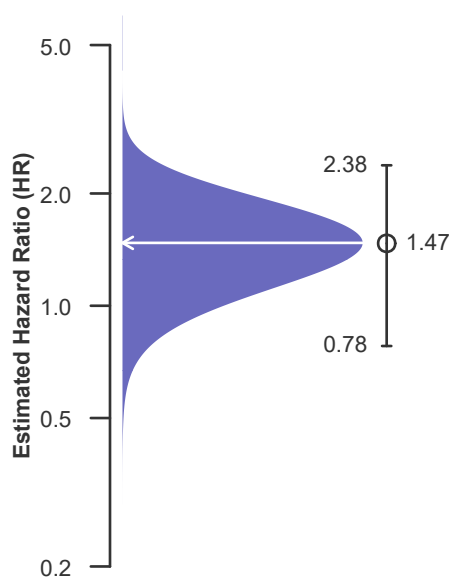
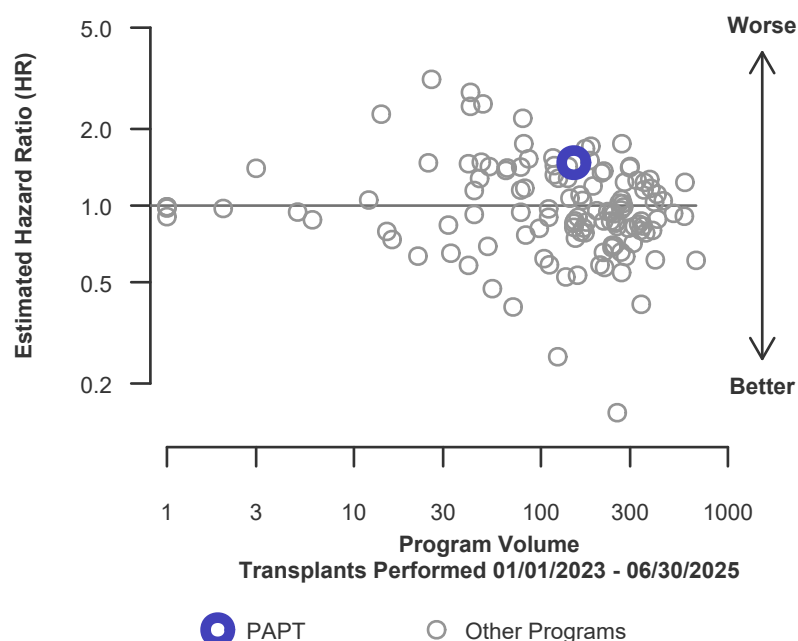


Figure C4D. Adult (18+) 90-Day deceased donor graft failure HR program comparison





C. Transplant Information

Table C6L. Adult (18+) 90-Day survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	201	1,349
Estimated probability of surviving with a functioning graft at 90 days & [95% CI] (unadjusted for patient and donor characteristics)	95.52% [92.71%-98.42%]	95.85% [94.79%-96.92%]
Expected probability of surviving with a functioning graft at 90 days (adjusted for patient and donor characteristics)	95.61%	--
Number of observed graft failures (including deaths) during the first 90 days after transplant	9	56
Number of expected graft failures (including deaths) during the first 90 days after transplant	9.40	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.48, 1.61]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.48, 1.61], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 52% reduced risk up to 61% increased risk.

Figure C3L. Adult (18+) 90-Day living donor graft failure HR estimate

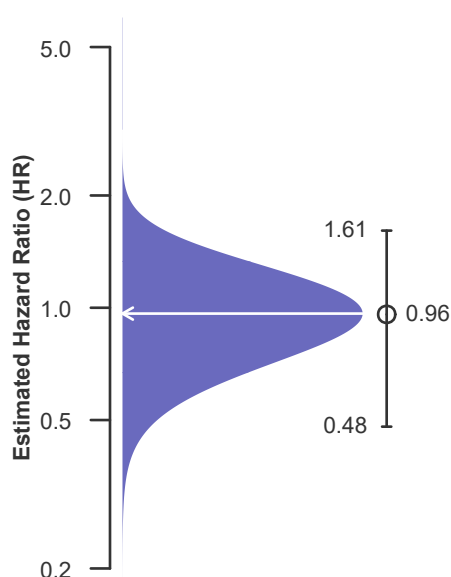
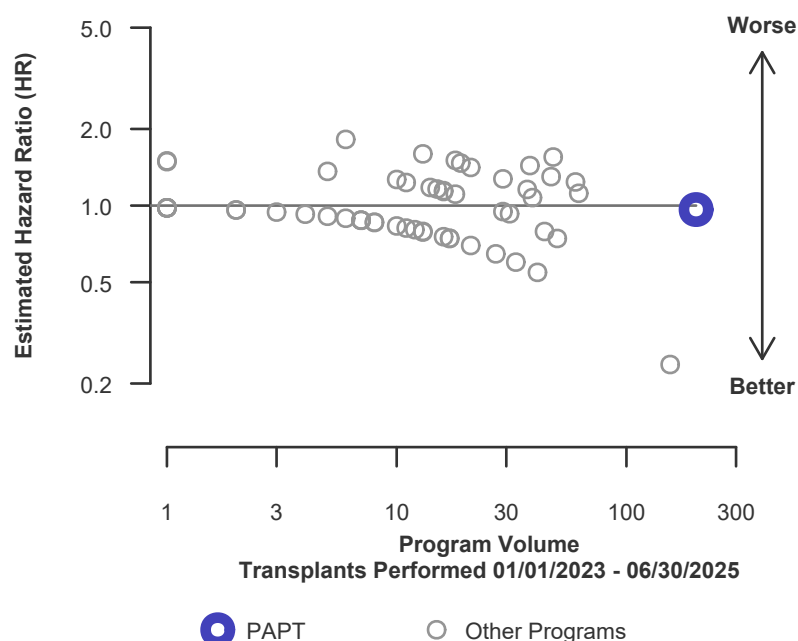


Figure C4L. Adult (18+) 90-Day living donor graft failure HR program comparison





C. Transplant Information

Table C7. Adult (18+) 1-year survival with a functioning graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	351	24,637
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	89.01% [85.66%-92.50%]	92.29% [91.94%-92.63%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	92.74%	--
Number of observed graft failures (including deaths) during the first year after transplant	36	1,798
Number of expected graft failures (including deaths) during the first year after transplant	24.33	--
Estimated hazard ratio*	1.44	--
95% credible interval for the hazard ratio**	[1.02, 1.94]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.02, 1.94], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 44% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 2% increased risk up to 94% increased risk.

Figure C5. Adult (18+) 1-year graft failure HR estimate

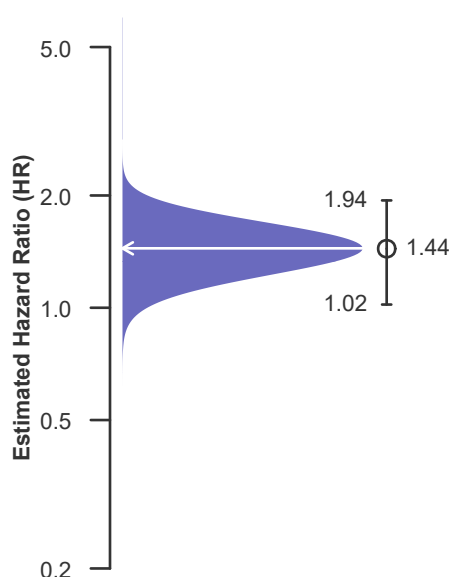
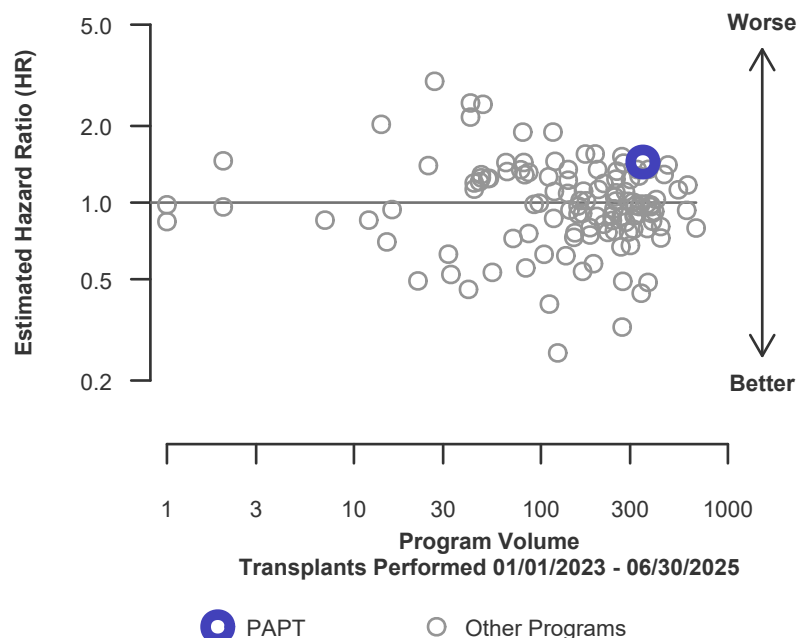


Figure C6. Adult (18+) 1-year graft failure HR program comparison





C. Transplant Information

Table C7D. Adult (18+) 1-year survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	150	23,288
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	88.96% [83.97%-94.24%]	92.21% [91.86%-92.57%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	92.06%	--
Number of observed graft failures (including deaths) during the first year after transplant	16	1,715
Number of expected graft failures (including deaths) during the first year after transplant	11.00	--
Estimated hazard ratio*	1.38	--
95% credible interval for the hazard ratio**	[0.82, 2.09]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.82, 2.09], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 38% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 18% reduced risk up to 109% increased risk.

Figure C5D. Adult (18+) 1-year deceased donor graft failure HR estimate

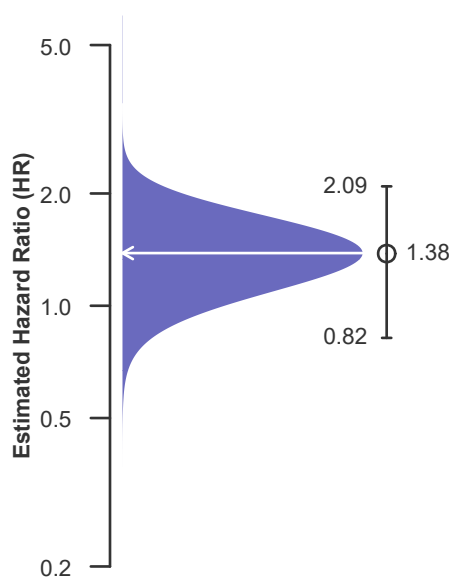
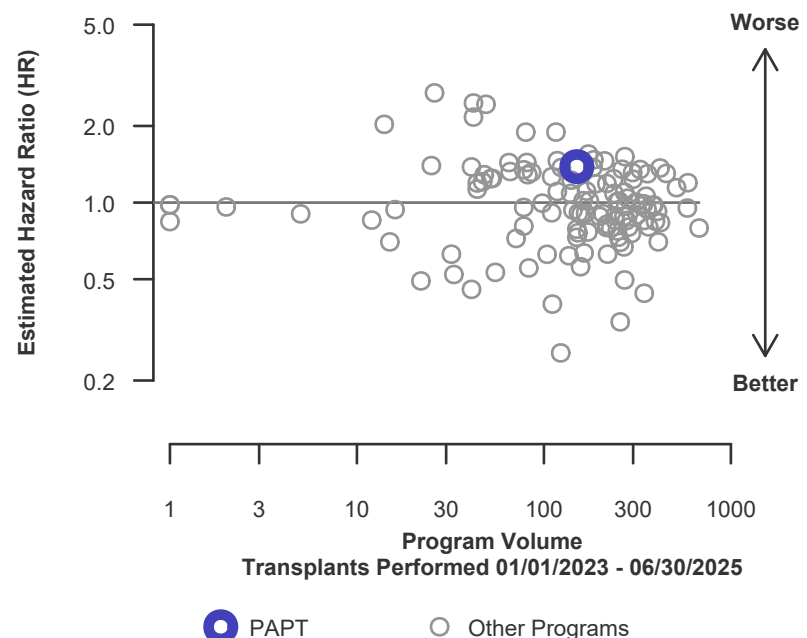


Figure C6D. Adult (18+) 1-year deceased donor graft failure HR program comparison





C. Transplant Information

Table C7L. Adult (18+) 1-year survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	201	1,349
Estimated probability of surviving with a functioning graft at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	89.05% [84.58%-93.75%]	93.55% [92.21%-94.91%]
Expected probability of surviving with a functioning graft at 1 year (adjusted for patient and donor characteristics)	93.25%	--
Number of observed graft failures (including deaths) during the first year after transplant	20	83
Number of expected graft failures (including deaths) during the first year after transplant	13.33	--
Estimated hazard ratio*	1.44	--
95% credible interval for the hazard ratio**	[0.90, 2.09]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.90, 2.09], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 44% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 10% reduced risk up to 109% increased risk.

Figure C5L. Adult (18+) 1-year living donor graft failure HR estimate

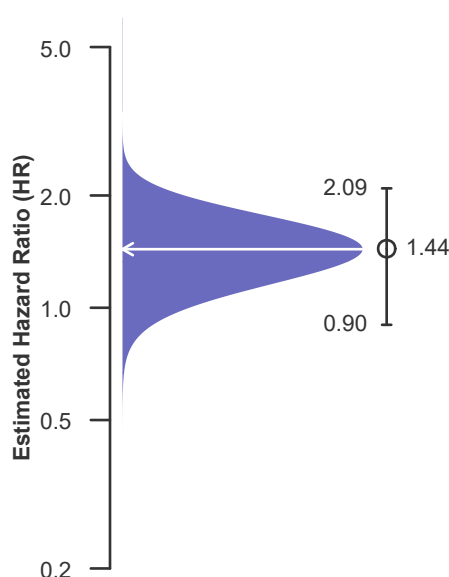
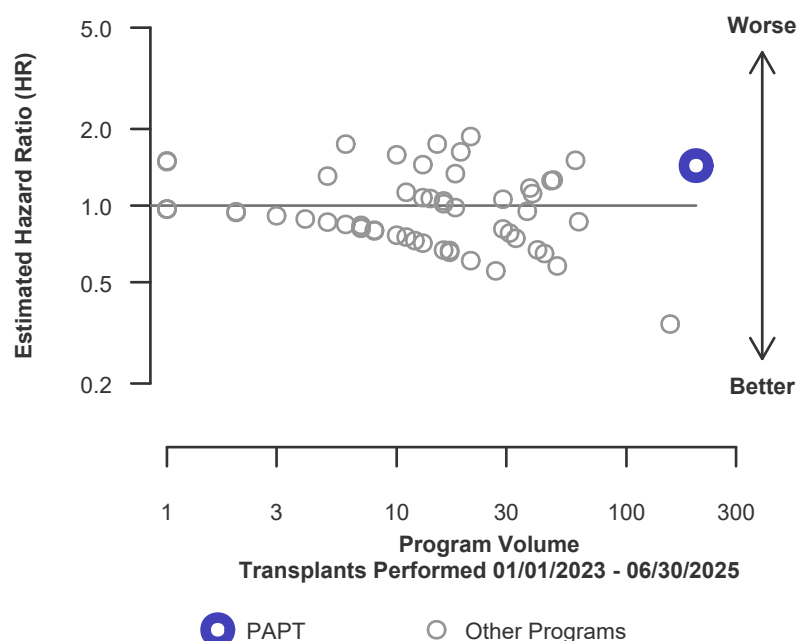


Figure C6L. Adult (18+) 1-year living donor graft failure HR program comparison





C. Transplant Information

Table C8. Adult (18+) 1-year Conditional survival with a functioning graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	331	23,551
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	94.39% [93.20%-95.60%]	96.54% [96.44%-96.65%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.08%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	16	712
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	8.11	--
Estimated hazard ratio*	1.78	--
95% credible interval for the hazard ratio**	[1.06, 2.69]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.06, 2.69], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 78% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 6% increased risk up to 169% increased risk.

Figure C7. Adult (18+) 1-year Conditional graft failure HR estimate

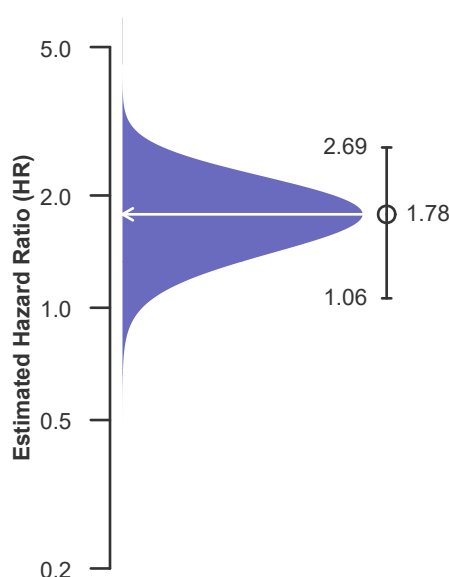
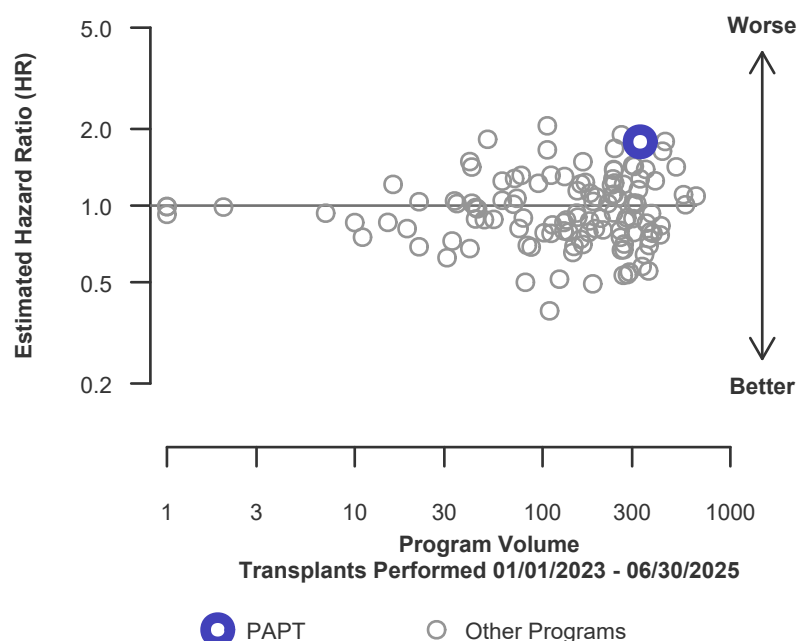


Figure C8. Adult (18+) 1-year Conditional graft failure HR program comparison





C. Transplant Information

Table C8D. Adult (18+) 1-year Conditional survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	139	22,258
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	96.00% [94.78%-97.22%]	96.48% [96.37%-96.59%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	96.46%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	5	685
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	4.19	--
Estimated hazard ratio*	1.13	--
95% credible interval for the hazard ratio**	[0.45, 2.11]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.45, 2.11], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 13% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 55% reduced risk up to 111% increased risk.

Figure C7D. Adult (18+) 1-year Conditional deceased donor graft failure HR estimate

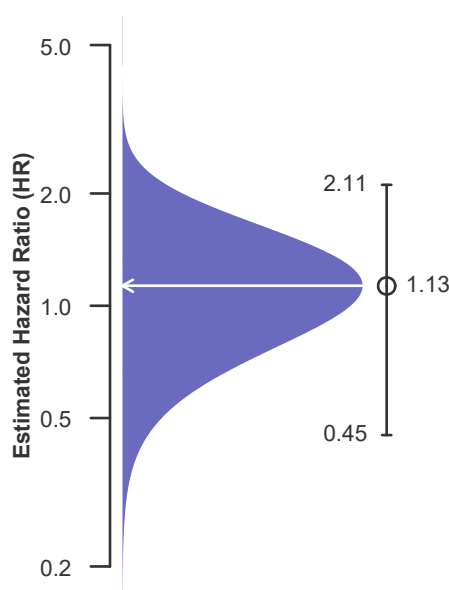
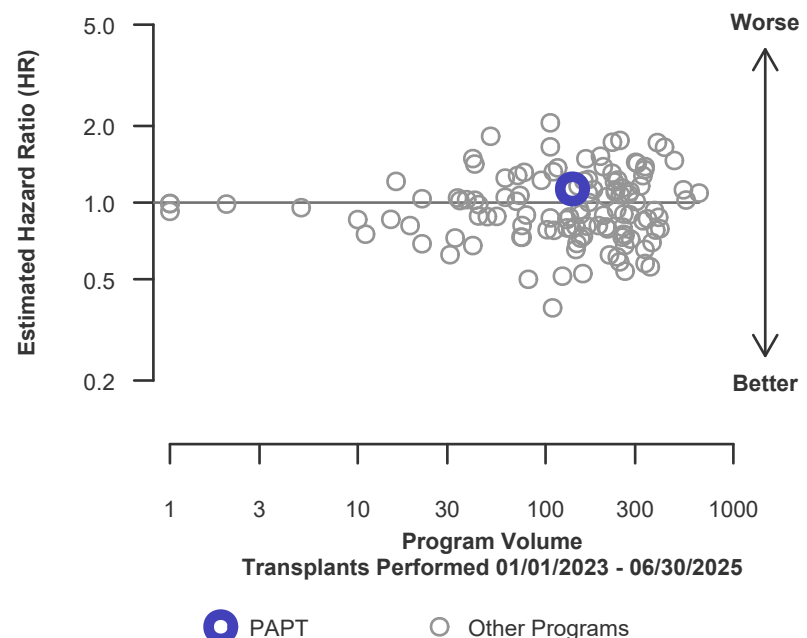


Figure C8D. Adult (18+) 1-year Conditional deceased donor graft failure HR program comparison





C. Transplant Information

Table C8L. Adult (18+) 1-year Conditional survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	192	1,293
Estimated probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 & [95% CI] (unadjusted for patient and donor characteristics)	93.22% [91.24%-95.25%]	97.60% [97.28%-97.93%]
Expected probability of surviving with a functioning graft at 1 year, among patients with a functioning graft at day 90 (adjusted for patient and donor characteristics)	97.54%	--
Number of observed graft failures (including deaths) from day 91 through day 365 after transplant	11	27
Number of expected graft failures (including deaths) from day 91 through day 365 after transplant	3.92	--
Estimated hazard ratio*	2.19	--
95% credible interval for the hazard ratio**	[1.17, 3.54]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.17, 3.54], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 119% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 17% increased risk up to 254% increased risk.

Figure C7L. Adult (18+) 1-year Conditional living donor graft failure HR estimate

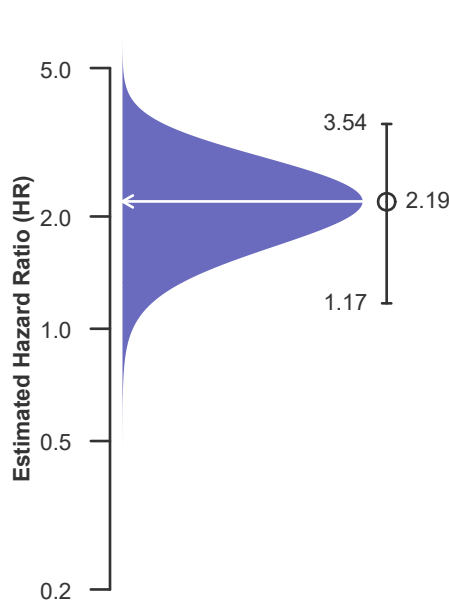
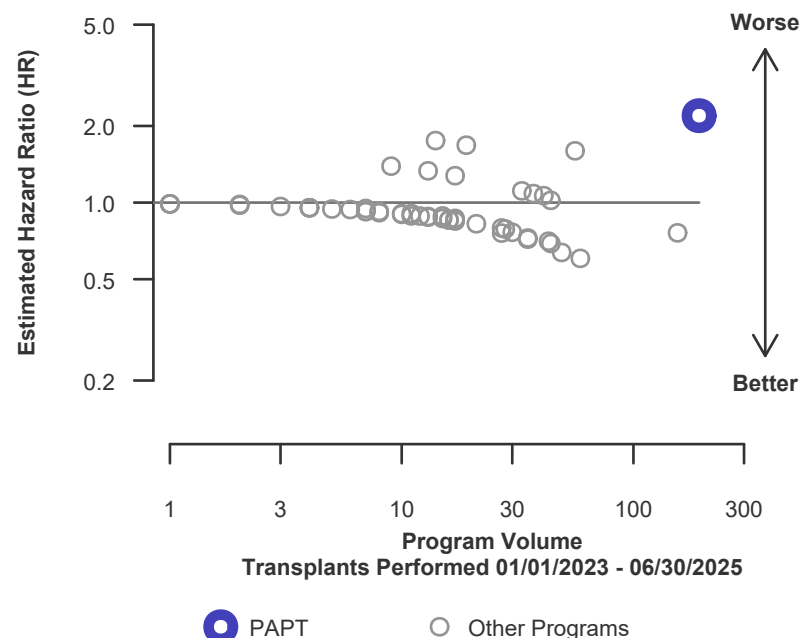


Figure C8L. Adult (18+) 1-year Conditional living donor graft failure HR program comparison





C. Transplant Information

Table C9. Adult (18+) 3-year survival with a functioning graft
Single organ transplants performed between 07/01/2020 and 12/31/2022
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	282	19,856
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	84.75% [80.66%-89.05%]	86.50% [86.02%-86.97%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	88.19%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	43	2,681
Number of expected graft failures (including deaths) during the first 3 years after transplant	32.34	--
Estimated hazard ratio*	1.31	--
95% credible interval for the hazard ratio**	[0.96, 1.72]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.96, 1.72], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 31% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 4% reduced risk up to 72% increased risk.

Figure C9. Adult (18+) 3-year graft failure HR estimate

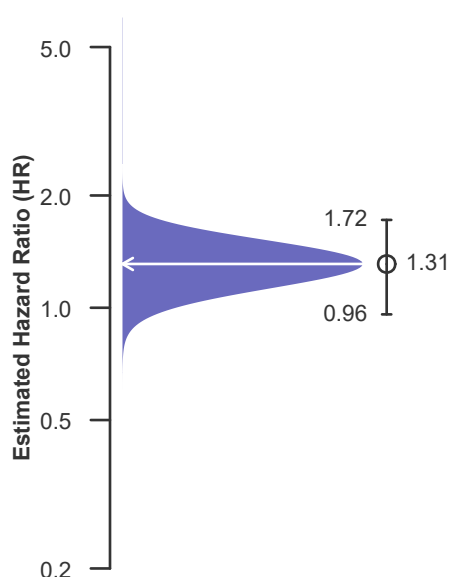
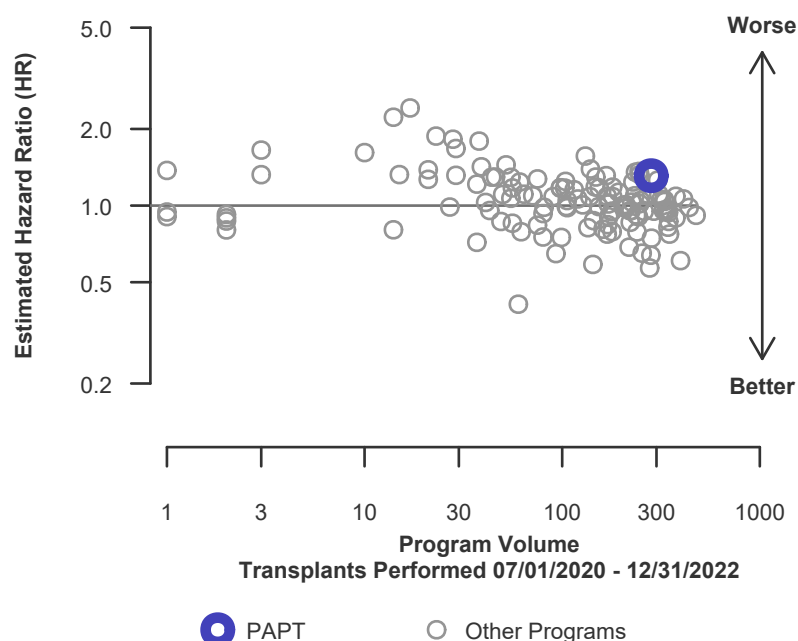


Figure C10. Adult (18+) 3-year graft failure HR program comparison





C. Transplant Information

Table C9D. Adult (18+) 3-year survival with a functioning deceased donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	99	18,615
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	77.78% [70.00%-86.41%]	86.34% [85.85%-86.83%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	86.91%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	22	2,543
Number of expected graft failures (including deaths) during the first 3 years after transplant	11.72	--
Estimated hazard ratio*	1.75	--
95% credible interval for the hazard ratio**	[1.12, 2.52]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.12, 2.52], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 75% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 12% increased risk up to 152% increased risk.

Figure C9D. Adult (18+) 3-year deceased donor graft failure HR estimate

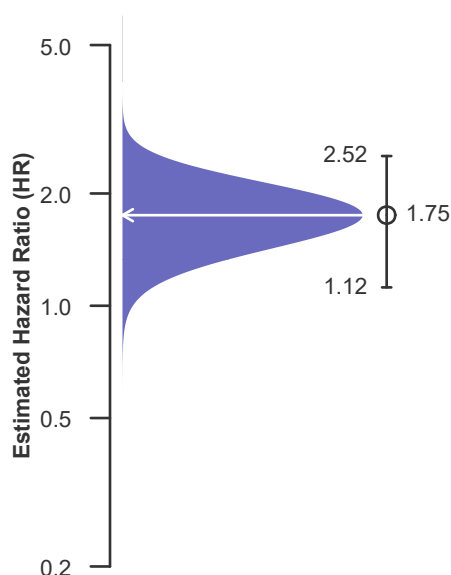
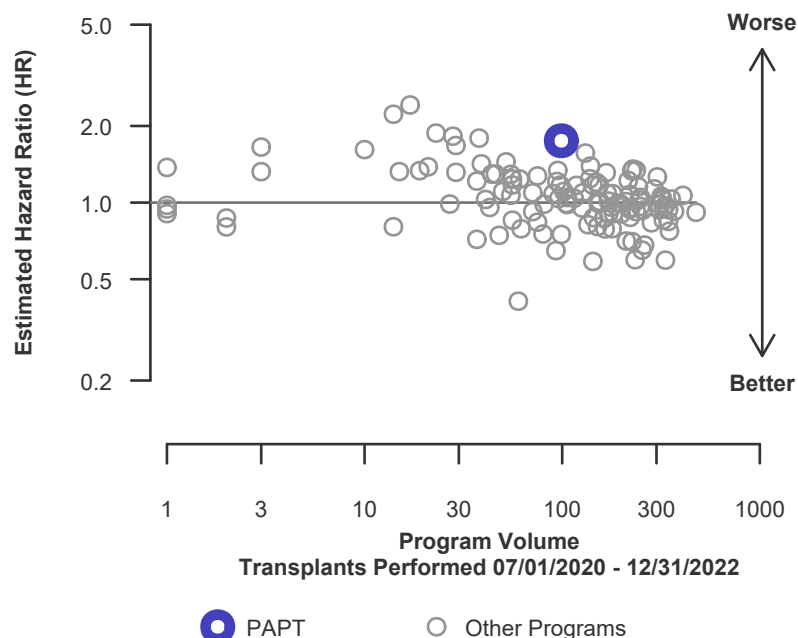


Figure C10D. Adult (18+) 3-year deceased donor graft failure HR program comparison





C. Transplant Information

Table C9L. Adult (18+) 3-year survival with a functioning living donor graft
Single organ transplants performed between 07/01/2020 and 12/31/2022
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	183	1,241
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	88.52% [84.03%-93.26%]	88.88% [87.15%-90.65%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	88.89%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	21	138
Number of expected graft failures (including deaths) during the first 3 years after transplant	20.62	--
Estimated hazard ratio*	1.02	--
95% credible interval for the hazard ratio**	[0.64, 1.47]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.64, 1.47], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 2% higher risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 36% reduced risk up to 47% increased risk.

Figure C9L. Adult (18+) 3-year living donor graft failure HR estimate

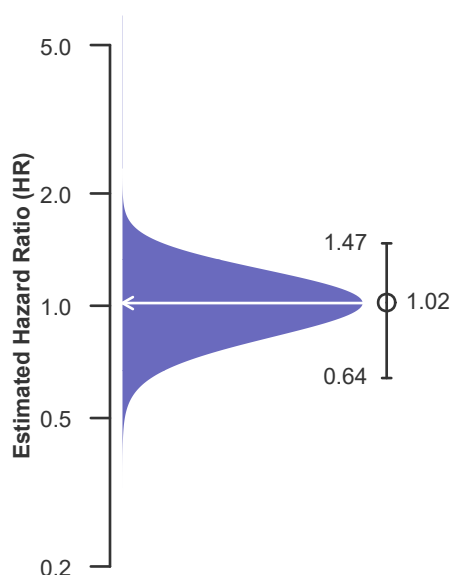
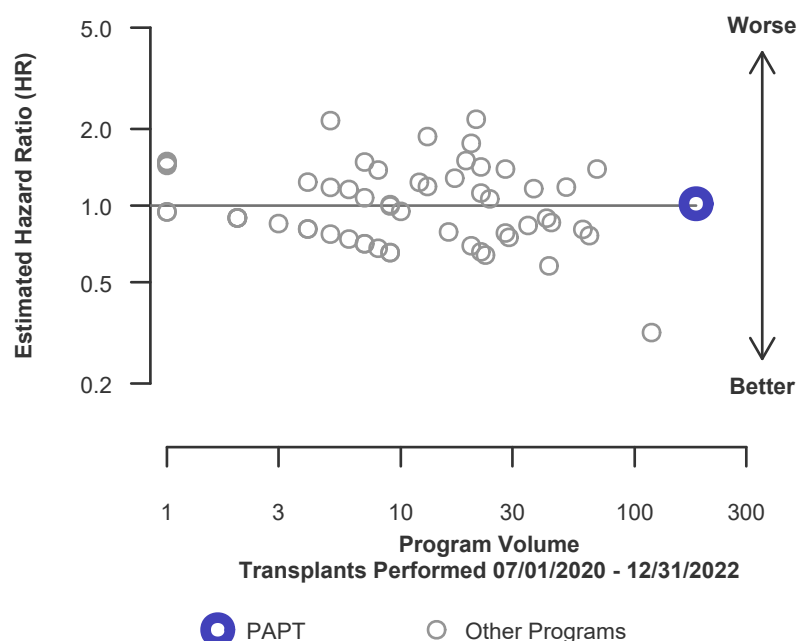


Figure C10L. Adult (18+) 3-year living donor graft failure HR program comparison





C. Transplant Information

Table C10. Pediatric (<18) 1-month survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C11. Pediatric (<18)
1-month graft failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C12. Pediatric (<18) 1-month graft failure HR program
comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C10D. Pediatric (<18) 1-month survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C11D. Pediatric (<18)
1-month deceased donor graft
failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C12D. Pediatric (<18) 1-month deceased donor graft
failure HR program comparison

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C10L. Pediatric (<18) 1-month survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C11L. Pediatric (<18)
1-month living donor graft failure
HR estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C12L. Pediatric (<18) 1-month living donor graft failure
HR program comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C11. Pediatric (<18) 90-Day survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C13. Pediatric (<18)
90-Day graft failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C14. Pediatric (<18) 90-Day graft failure HR program
comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C11D. Pediatric (<18) 90-Day survival with a functioning deceased donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C13D. Pediatric (<18) 90-Day deceased donor graft failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C14D. Pediatric (<18) 90-Day deceased donor graft failure HR program comparison

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C11L. Pediatric (<18) 90-Day survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C13L. Pediatric (<18)
90-Day living donor graft failure
HR estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C14L. Pediatric (<18) 90-Day living donor graft failure
HR program comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C12. Pediatric (<18) 1-year survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C15. Pediatric (<18) 1-year
graft failure HR estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C16. Pediatric (<18) 1-year graft failure HR program
comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C12D. Pediatric (<18) 1-year survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C15D. Pediatric (<18)
1-year deceased donor graft
failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C16D. Pediatric (<18) 1-year deceased donor graft
failure HR program comparison

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C12L. Pediatric (<18) 1-year survival with a functioning living donor graft

Single organ transplants performed between 01/01/2023 and 06/30/2025

Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C15L. Pediatric (<18)
1-year living donor graft failure
HR estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C16L. Pediatric (<18) 1-year living donor graft failure
HR program comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C13. Pediatric (<18) 1-year Conditional survival with a functioning graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C17. Pediatric (<18) 1-year
Conditional graft failure HR
estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C18. Pediatric (<18) 1-year Conditional graft failure HR
program comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C13D. Pediatric (<18) 1-year Conditional survival with a functioning deceased donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C17D. Pediatric (<18)
1-year Conditional deceased
donor graft failure HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C18D. Pediatric (<18) 1-year Conditional deceased
donor graft failure HR program comparison

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C13L. Pediatric (<18) 1-year Conditional survival with a functioning living donor graft
Single organ transplants performed between 01/01/2023 and 06/30/2025
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C17L. Pediatric (<18)
1-year Conditional living donor
graft failure HR estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C18L. Pediatric (<18) 1-year Conditional living donor
graft failure HR program comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C14. Pediatric (<18) 3-year survival with a functioning graft
Single organ transplants performed between 07/01/2020 and 12/31/2022
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	1	1,230
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	89.67% [87.99%-91.39%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	91.94%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	127
Number of expected graft failures (including deaths) during the first 3 years after transplant	0.08	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.67]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.67], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 88% reduced risk up to 167% increased risk.

Figure C19. Pediatric (<18) 3-year graft failure HR estimate

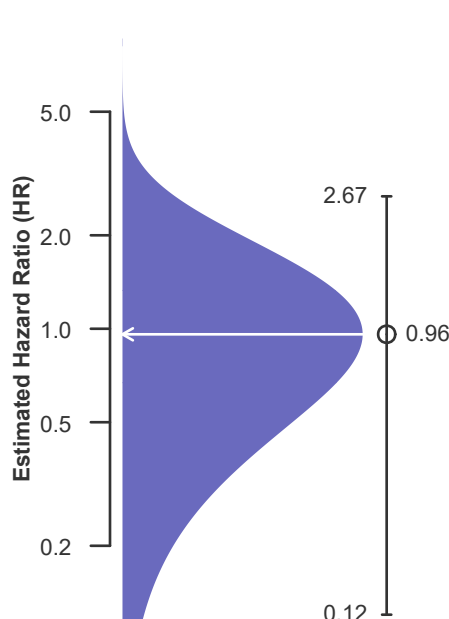
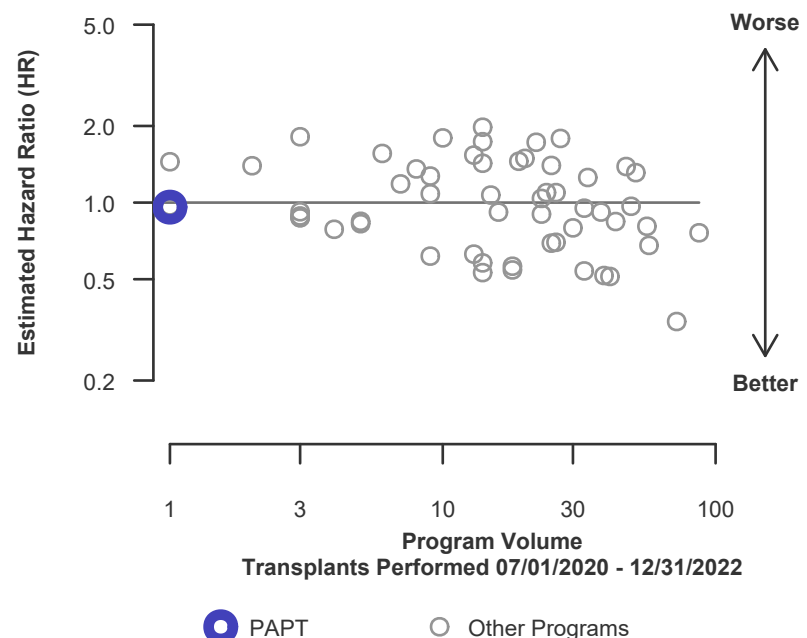


Figure C20. Pediatric (<18) 3-year graft failure HR program comparison





C. Transplant Information

Table C14D. Pediatric (<18) 3-year survival with a functioning deceased donor graft
Single organ transplants performed between 07/01/2020 and 12/31/2022
Deaths and retransplants are considered graft failures

This center did not perform any
transplants relevant to
this table during
07/01/2020-12/31/2022

Figure C19D. Pediatric (<18)
3-year deceased donor graft
failure HR estimate

This center did not perform any
transplants relevant to
this figure during
07/01/2020-12/31/2022

Figure C20D. Pediatric (<18) 3-year deceased donor graft
failure HR program comparison

This center did not perform any
transplants relevant to
this figure during
07/01/2020-12/31/2022



C. Transplant Information

Table C14L. Pediatric (<18) 3-year survival with a functioning living donor graft

Single organ transplants performed between 07/01/2020 and 12/31/2022

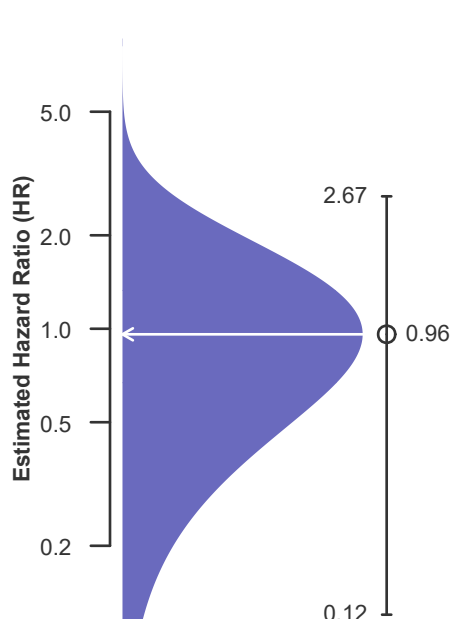
Deaths and retransplants are considered graft failures

	PAPT	U.S.
Number of transplants evaluated	1	198
Estimated probability of surviving with a functioning graft at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	91.92% [88.20%-95.79%]
Expected probability of surviving with a functioning graft at 3 years (adjusted for patient and donor characteristics)	91.94%	--
Number of observed graft failures (including deaths) during the first 3 years after transplant	0	16
Number of expected graft failures (including deaths) during the first 3 years after transplant	0.08	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.67]	--

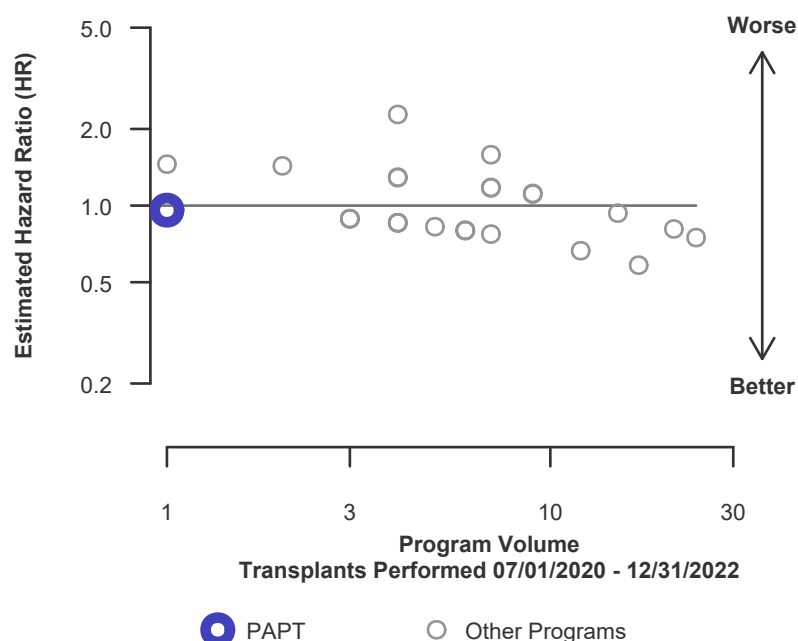
* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected graft failure rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected graft failure rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's graft failure rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.67], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of graft failure compared to an average program, but PAPT's performance could plausibly range from 88% reduced risk up to 167% increased risk.

**Figure C19L. Pediatric (<18)
3-year living donor graft failure
HR estimate**



**Figure C20L. Pediatric (<18) 3-year living donor graft failure
HR program comparison**





C. Transplant Information

Table C15. Adult (18+) 1-month patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	335	23,811
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	98.81% [97.65%-99.98%]	98.47% [98.32%-98.63%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	98.36%	--
Number of observed deaths during the first month after transplant	4	364
Number of expected deaths during the first month after transplant	5.70	--
Estimated hazard ratio*	0.78	--
95% credible interval for the hazard ratio**	[0.29, 1.52]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.29, 1.52], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 22% lower risk of patient death compared to an average program, but PAPT's performance could plausibly range from 71% reduced risk up to 52% increased risk.

Figure C21. Adult (18+) 1-month patient death HR estimate

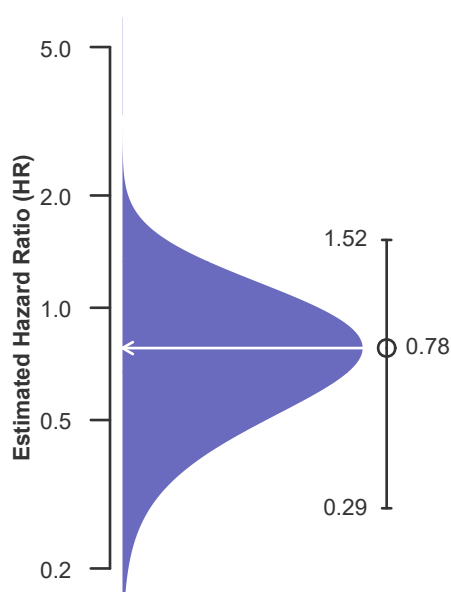
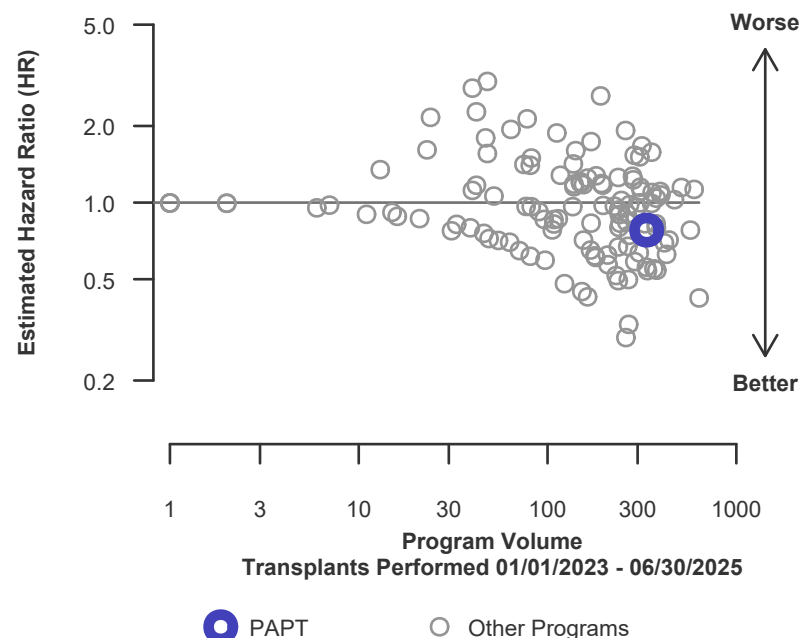


Figure C22. Adult (18+) 1-month patient death HR program comparison





C. Transplant Information

Table C15D. Adult (18+) 1-month patient survival (deceased donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	138	22,475
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	98.55% [96.58%-100.00%]	98.44% [98.28%-98.60%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	98.44%	--
Number of observed deaths during the first month after transplant	2	351
Number of expected deaths during the first month after transplant	2.17	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.26, 2.10]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.26, 2.10], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of patient death compared to an average program, but PAPT's performance could plausibly range from 74% reduced risk up to 110% increased risk.

Figure C21D. Adult (18+) 1-month patient death HR estimate (deceased donor grafts)

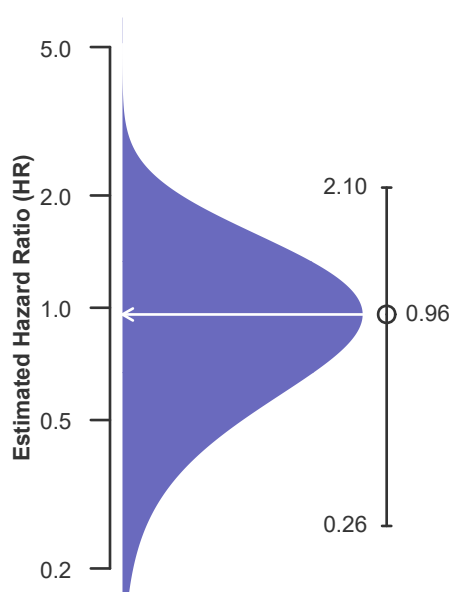
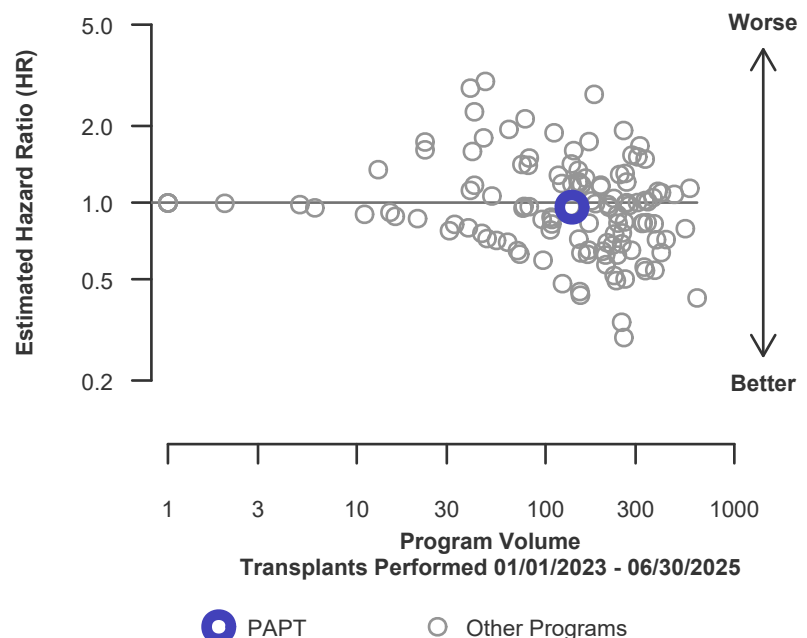


Figure C22D. Adult (18+) 1-month patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C15L. Adult (18+) 1-month patient survival (living donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	197	1,336
Estimated probability of surviving at 1 month & [95% CI] (unadjusted for patient and donor characteristics)	98.98% [97.59%-100.00%]	99.03% [98.50%-99.55%]
Expected probability of surviving at 1 month (adjusted for patient and donor characteristics)	98.31%	--
Number of observed deaths during the first month after transplant	2	13
Number of expected deaths during the first month after transplant	3.53	--
Estimated hazard ratio*	0.72	--
95% credible interval for the hazard ratio**	[0.20, 1.59]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.20, 1.59], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 28% lower risk of patient death compared to an average program, but PAPT's performance could plausibly range from 80% reduced risk up to 59% increased risk.

Figure C21L. Adult (18+) 1-month patient death HR estimate (living donor grafts)

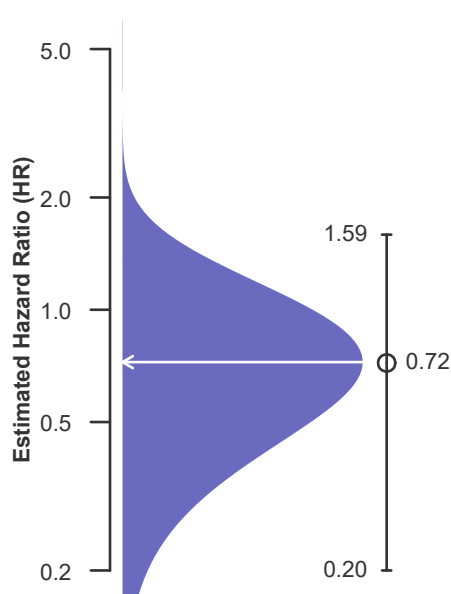
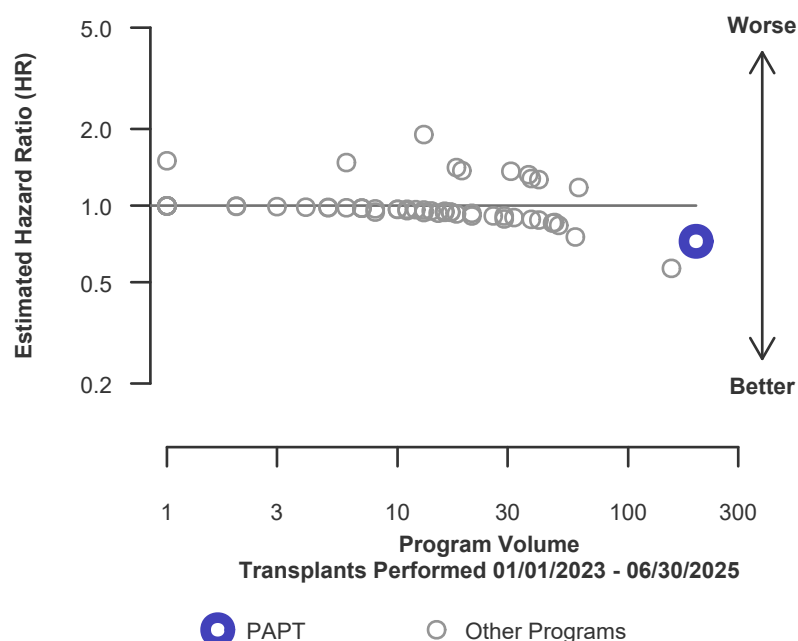


Figure C22L. Adult (18+) 1-month patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C16. Adult (18+) 1-year patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	335	23,811
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	92.25% [89.29%-95.30%]	94.27% [93.97%-94.58%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	93.98%	--
Number of observed deaths during the first year after transplant	24	1,277
Number of expected deaths during the first year after transplant	18.51	--
Estimated hazard ratio*	1.27	--
95% credible interval for the hazard ratio**	[0.83, 1.80]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.83, 1.80], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 27% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 17% reduced risk up to 80% increased risk.

Figure C23. Adult (18+) 1-year patient death HR estimate

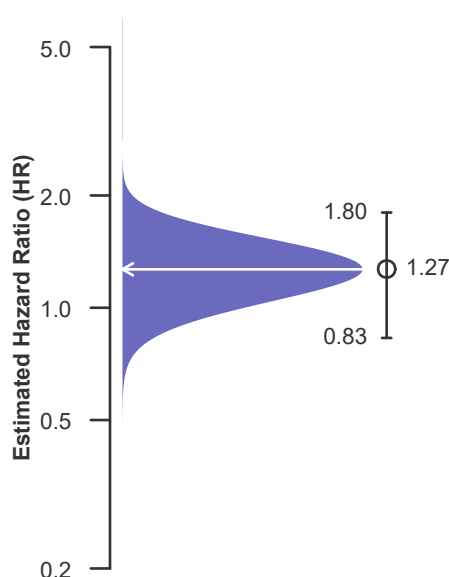
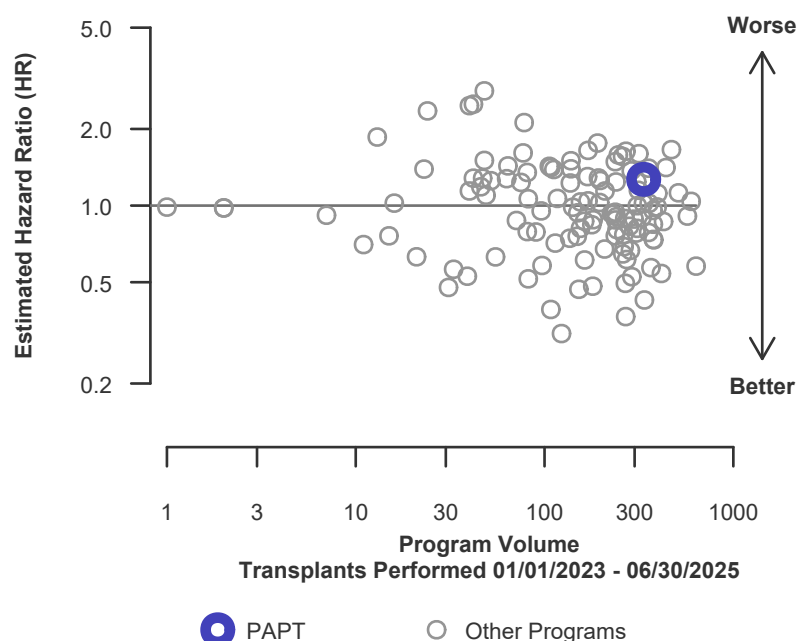


Figure C24. Adult (18+) 1-year patient death HR program comparison





C. Transplant Information

Table C16D. Adult (18+) 1-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	138	22,475
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	94.00% [90.03%-98.14%]	94.16% [93.84%-94.48%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	94.26%	--
Number of observed deaths during the first year after transplant	8	1,230
Number of expected deaths during the first year after transplant	7.37	--
Estimated hazard ratio*	1.07	--
95% credible interval for the hazard ratio**	[0.51, 1.82]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.51, 1.82], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 7% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 49% reduced risk up to 82% increased risk.

Figure C23D. Adult (18+) 1-year patient death HR estimate (deceased donor grafts)

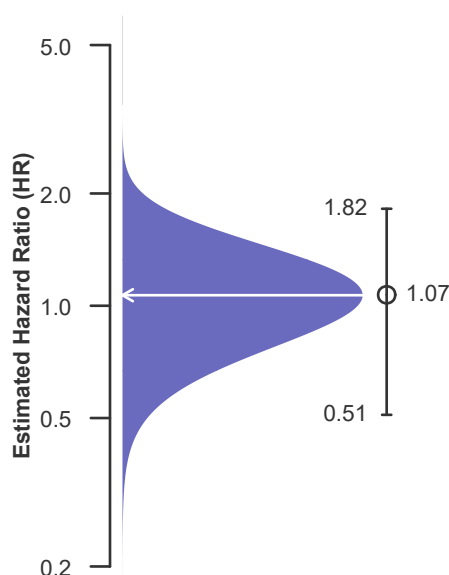
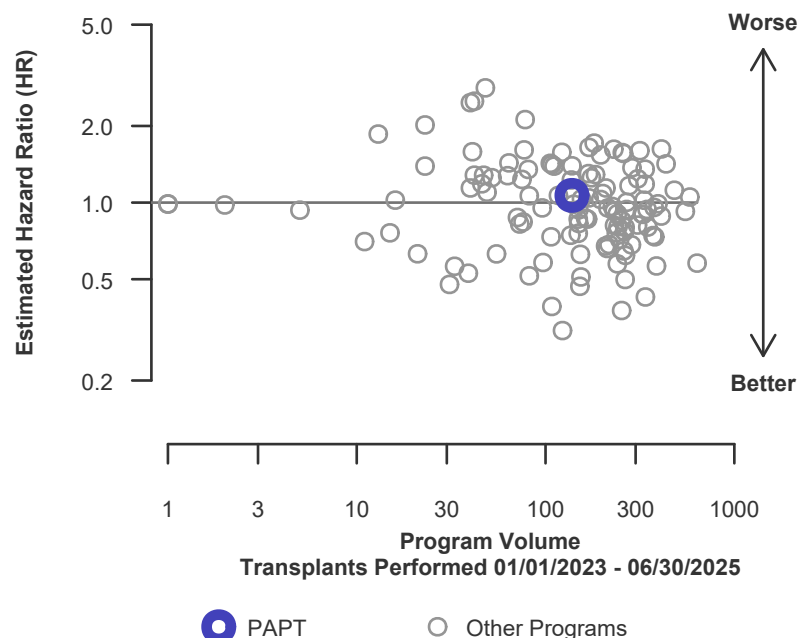


Figure C24D. Adult (18+) 1-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C16L. Adult (18+) 1-year patient survival (living donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	197	1,336
Estimated probability of surviving at 1 year & [95% CI] (unadjusted for patient and donor characteristics)	91.02% [86.88%-95.36%]	96.21% [95.14%-97.28%]
Expected probability of surviving at 1 year (adjusted for patient and donor characteristics)	93.78%	--
Number of observed deaths during the first year after transplant	16	47
Number of expected deaths during the first year after transplant	11.14	--
Estimated hazard ratio*	1.37	--
95% credible interval for the hazard ratio**	[0.81, 2.07]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.81, 2.07], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 37% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 19% reduced risk up to 107% increased risk.

Figure C23L. Adult (18+) 1-year patient death HR estimate (living donor grafts)

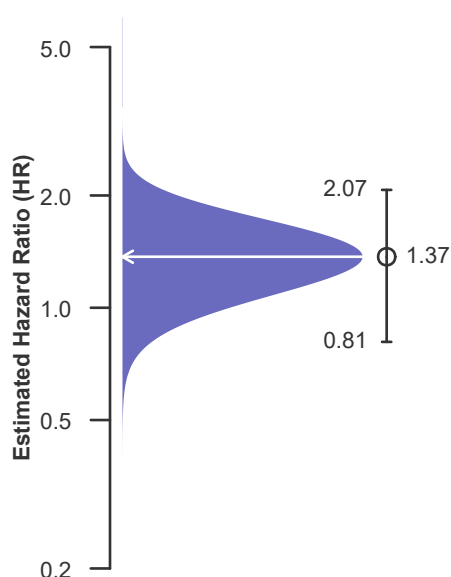
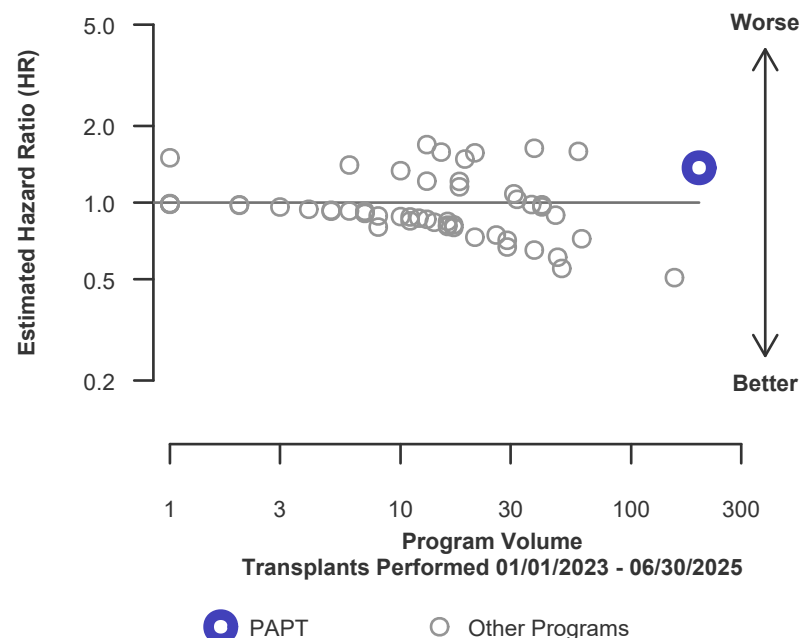


Figure C24L. Adult (18+) 1-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C17. Adult (18+) 3-year patient survival

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	274	19,158
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	85.77% [81.73%-90.00%]	88.78% [88.33%-89.23%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	89.86%	--
Number of observed deaths during the first 3 years after transplant	39	2,150
Number of expected deaths during the first 3 years after transplant	26.83	--
Estimated hazard ratio*	1.42	--
95% credible interval for the hazard ratio**	[1.02, 1.89]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.02, 1.89], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 42% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 2% increased risk up to 89% increased risk.

Figure C25. Adult (18+) 3-year patient death HR estimate

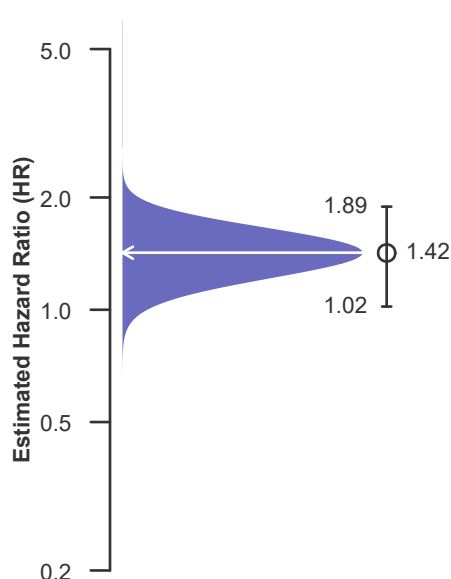
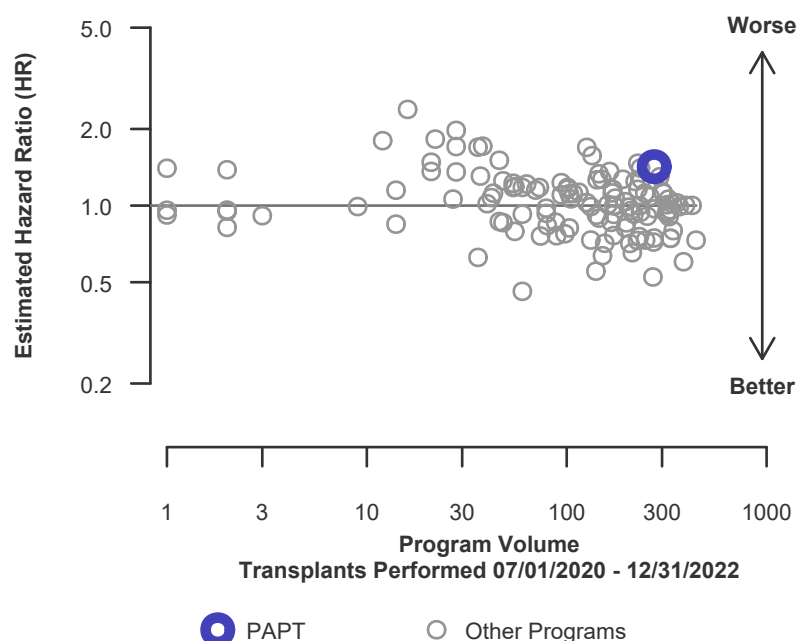


Figure C26. Adult (18+) 3-year patient death HR program comparison





C. Transplant Information

Table C17D. Adult (18+) 3-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	96	17,930
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	80.21% [72.62%-88.59%]	88.57% [88.10%-89.03%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	89.57%	--
Number of observed deaths during the first 3 years after transplant	19	2,050
Number of expected deaths during the first 3 years after transplant	9.36	--
Estimated hazard ratio*	1.85	--
95% credible interval for the hazard ratio**	[1.14, 2.72]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [1.14, 2.72], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 85% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 14% increased risk up to 172% increased risk.

Figure C25D. Adult (18+) 3-year patient death HR estimate (deceased donor grafts)

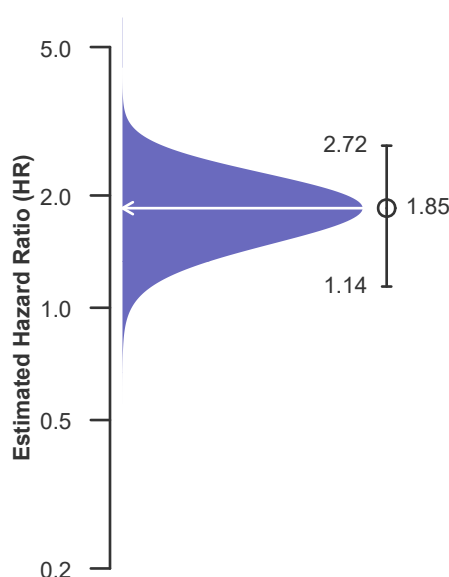
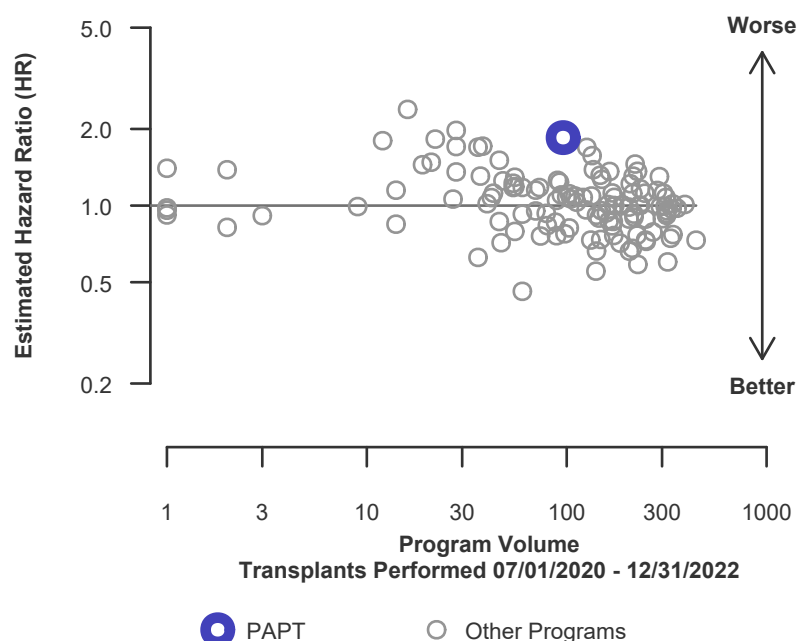


Figure C26D. Adult (18+) 3-year patient death HR program comparison (deceased donor grafts)





C. Transplant Information

Table C17L. Adult (18+) 3-year patient survival (living donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	178	1,228
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	88.76% [84.24%-93.53%]	91.86% [90.34%-93.40%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	90.01%	--
Number of observed deaths during the first 3 years after transplant	20	100
Number of expected deaths during the first 3 years after transplant	17.47	--
Estimated hazard ratio*	1.13	--
95% credible interval for the hazard ratio**	[0.71, 1.65]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.71, 1.65], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 13% higher risk of patient death compared to an average program, but PAPT's performance could plausibly range from 29% reduced risk up to 65% increased risk.

Figure C25L. Adult (18+) 3-year patient death HR estimate (living donor grafts)

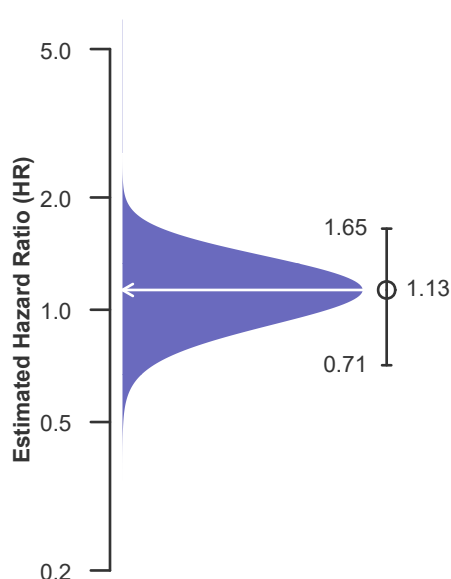
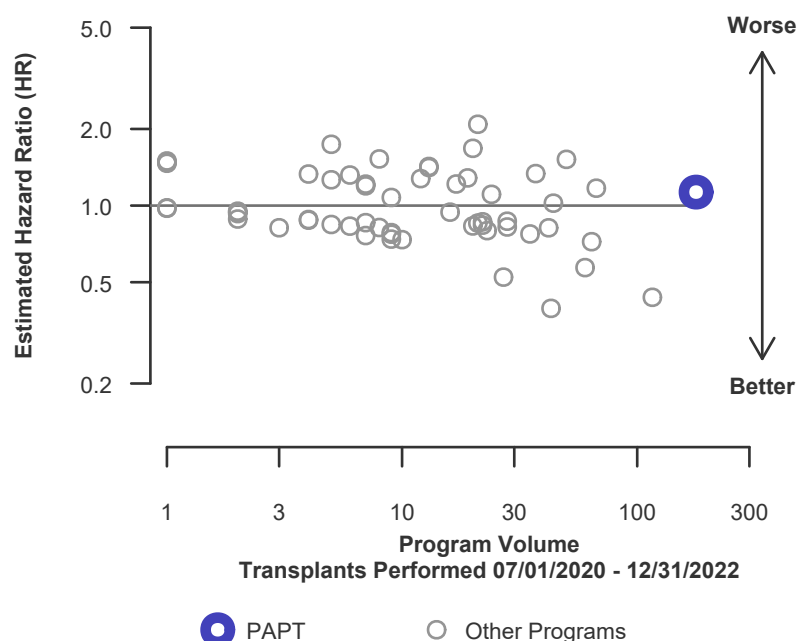


Figure C26L. Adult (18+) 3-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C18. Pediatric (<18) 1-month patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C27. Pediatric (<18)
1-month patient death HR
estimate**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C28. Pediatric (<18) 1-month patient death HR program
comparison**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C18D. Pediatric (<18) 1-month patient survival (deceased donor graft recipients)

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C27D. Pediatric (<18)
1-month patient death HR
estimate (deceased donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C28D. Pediatric (<18) 1-month patient death HR
program comparison (deceased donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C18L. Pediatric (<18) 1-month patient survival (living donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C27L. Pediatric (<18)
1-month patient death HR
estimate (living donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C28L. Pediatric (<18) 1-month patient death HR
program comparison (living donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C19. Pediatric (<18) 1-year patient survival

Single organ transplants performed between 01/01/2023 and 06/30/2025

Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C29. Pediatric (<18) 1-year patient death HR estimate

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C30. Pediatric (<18) 1-year patient death HR program comparison

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C19D. Pediatric (<18) 1-year patient survival (deceased donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

Figure C29D. Pediatric (<18)
1-year patient death HR estimate
(deceased donor grafts)

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

Figure C30D. Pediatric (<18) 1-year patient death HR program
comparison (deceased donor grafts)

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C19L. Pediatric (<18) 1-year patient survival (living donor graft recipients)
Single organ transplants performed between 01/01/2023 and 06/30/2025
Retransplants excluded

This center did not perform any
transplants relevant to
this table during
01/01/2023-06/30/2025

**Figure C29L. Pediatric (<18)
1-year patient death HR estimate
(living donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025

**Figure C30L. Pediatric (<18) 1-year patient death HR program
comparison (living donor grafts)**

This center did not perform any
transplants relevant to
this figure during
01/01/2023-06/30/2025



C. Transplant Information

Table C20. Pediatric (<18) 3-year patient survival

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	1	1,161
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	92.94% [91.48%-94.42%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	92.88%	--
Number of observed deaths during the first 3 years after transplant	0	82
Number of expected deaths during the first 3 years after transplant	0.07	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.69]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.69], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of patient death compared to an average program, but PAPT's performance could plausibly range from 88% reduced risk up to 169% increased risk.

Figure C31. Pediatric (<18) 3-year patient death HR estimate

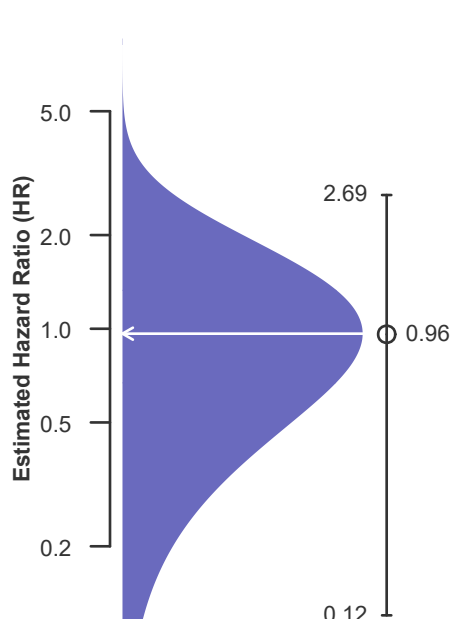
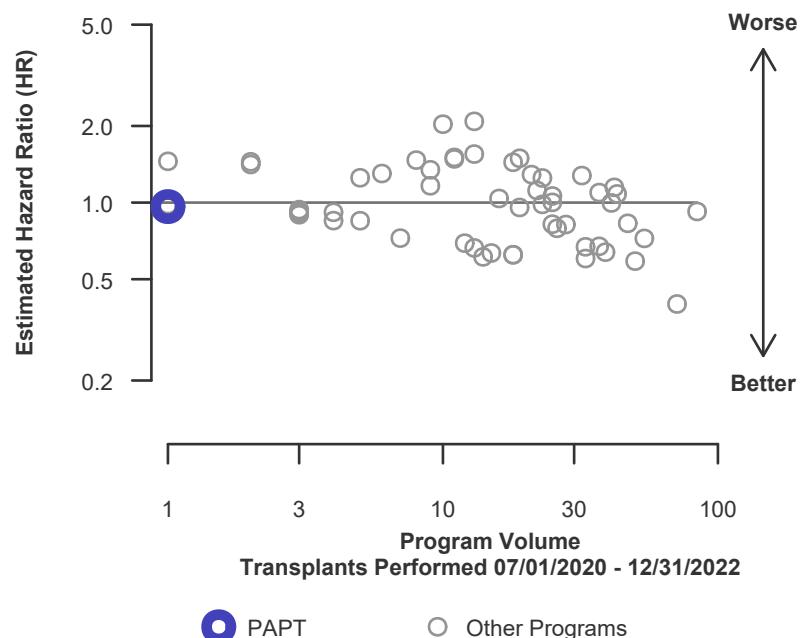


Figure C32. Pediatric (<18) 3-year patient death HR program comparison





C. Transplant Information

Table C20D. Pediatric (<18) 3-year patient survival (deceased donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

This center did not perform any
transplants relevant to
this table during
07/01/2020-12/31/2022

Figure C31D. Pediatric (<18) 3-year patient death HR estimate (deceased donor grafts)

This center did not perform any
transplants relevant to
this figure during
07/01/2020-12/31/2022

Figure C32D. Pediatric (<18) 3-year patient death HR program comparison (deceased donor grafts)

This center did not perform any
transplants relevant to
this figure during
07/01/2020-12/31/2022



C. Transplant Information

Table C20L. Pediatric (<18) 3-year patient survival (living donor graft recipients)

Single organ transplants performed between 07/01/2020 and 12/31/2022

Retransplants excluded

	PAPT	U.S.
Number of transplants evaluated	1	196
Estimated probability of surviving at 3 years & [95% CI] (unadjusted for patient and donor characteristics)	100.00% [100.00%-100.00%]	92.86% [89.32%-96.53%]
Expected probability of surviving at 3 years (adjusted for patient and donor characteristics)	92.88%	--
Number of observed deaths during the first 3 years after transplant	0	14
Number of expected deaths during the first 3 years after transplant	0.07	--
Estimated hazard ratio*	0.96	--
95% credible interval for the hazard ratio**	[0.12, 2.69]	--

* The hazard ratio provides an estimate of how University of Pittsburgh Medical Center's results compare with what was expected based on modeling the transplant outcomes from all U.S. programs. A ratio above 1 indicates higher than expected patient death rates (e.g., a hazard ratio of 1.5 would indicate 50% higher risk), and a ratio below 1 indicates lower than expected patient death rates (e.g., a hazard ratio of 0.75 would indicate 25% lower risk). If PAPT's patient death rate were precisely the expected rate, the estimated hazard ratio would be 1.0.

** The 95% credible interval, [0.12, 2.69], indicates the location of PAPT's true hazard ratio with 95% probability. The best estimate is 4% lower risk of patient death compared to an average program, but PAPT's performance could plausibly range from 88% reduced risk up to 169% increased risk.

Figure C31L. Pediatric (<18) 3-year patient death HR estimate (living donor grafts)

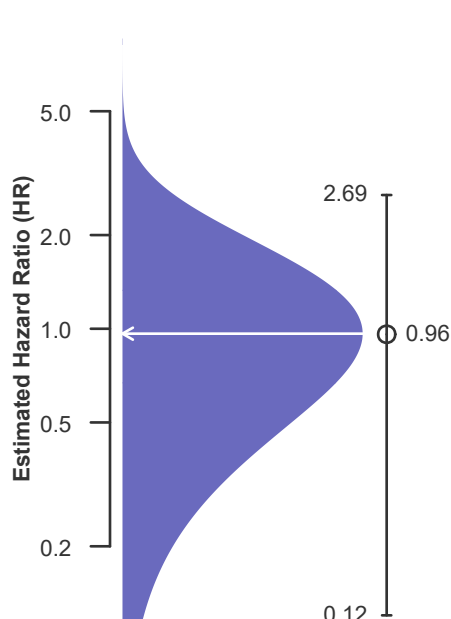
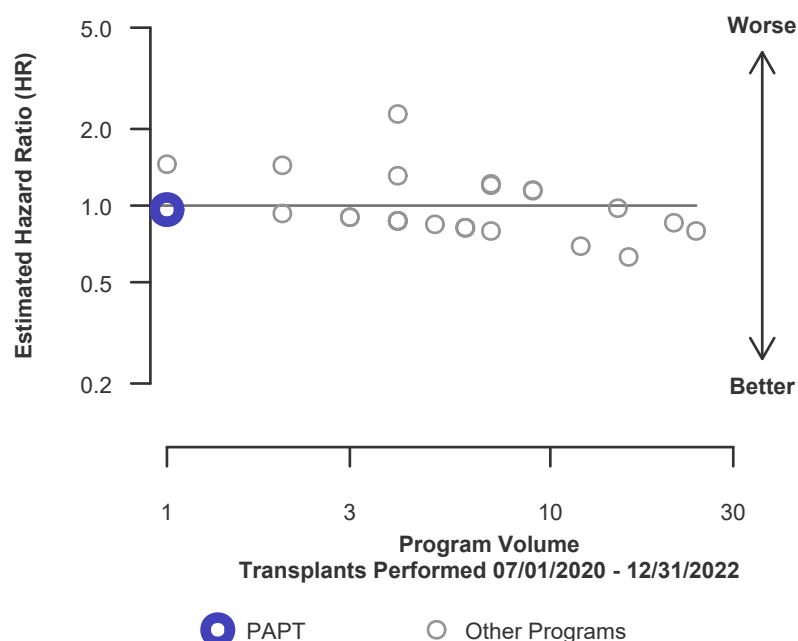


Figure C32L. Pediatric (<18) 3-year patient death HR program comparison (living donor grafts)





C. Transplant Information

Table C21. Multi-organ transplant graft survival: 01/01/2023 - 06/30/2025

Adult (18+) Transplants

Transplant Type	First-Year Outcomes					
	Transplants Performed		Liver Graft Failures		Estimated Liver Graft Survival	
	PAPT-TX1	USA	PAPT-TX1	USA	PAPT-TX1	USA
Kidney-Liver (Two Living Donors)	1	1	0	0	100.0%	100.0%
Kidney-Liver	2	1,984	0	186	100.0%	90.6%
Liver-Heart	6	183	1	36	83.3%	80.3%
Pancreas-Liver-Intestine	2	44	1	18	50.0%	59.1%

Pediatric (<18) Transplants

Transplant Type	First-Year Outcomes					
	Transplants Performed		Liver Graft Failures		Estimated Liver Graft Survival	
	PAPT-TX1	USA	PAPT-TX1	USA	PAPT-TX1	USA
Pancreas-Liver-Intestine	1	47	1	12	0.0%	74.5%

Table C22. Multi-organ transplant patient survival: 01/01/2023 - 06/30/2025

Adult (18+) Transplants

Transplant Type	First-Year Outcomes					
	Transplants Performed		Patient Deaths		Estimated Patient Survival	
	PAPT-TX1	USA	PAPT-TX1	USA	PAPT-TX1	USA
Kidney-Liver (Two Living Donors)	1	1	0	0	100.0%	100.0%
Kidney-Liver	2	1,984	0	175	100.0%	91.2%
Liver-Heart	6	183	1	35	83.3%	80.9%
Pancreas-Liver-Intestine	2	44	1	17	50.0%	61.4%

Pediatric (<18) Transplants

Transplant Type	First-Year Outcomes					
	Transplants Performed		Patient Deaths		Estimated Patient Survival	
	PAPT-TX1	USA	PAPT-TX1	USA	PAPT-TX1	USA
Pancreas-Liver-Intestine	1	47	1	9	0.0%	80.9%



D. Living Donor Information

Table D1. Living donor summary: 01/01/2023 - 12/31/2025

Living Donor Follow-Up	This Center			United States		
	01/2023- 12/2023	01/2024- 12/2024	01/2025- 06/2025	01/2023- 12/2023	01/2024- 12/2024	01/2025- 06/2025
Number of Living Donors	89	88	45	652	599	325
6-Month Follow-Up						
Donors due for follow-up	89	88	45	652	599	325
Timely clinical data	65 73.0%	72 81.8%	40 88.9%	550 84.4%	517 86.3%	274 84.3%
Timely lab data	66 74.2%	70 79.5%	41 91.1%	562 86.2%	521 87.0%	284 87.4%
12-Month Follow-Up						
Donors due for follow-up	89	88		652	599	
Timely clinical data	70 78.7%	67 76.1%		523 80.2%	484 80.8%	
Timely lab data	71 79.8%	67 76.1%		531 81.4%	496 82.8%	
24-Month Follow-Up						
Donors due for follow-up	89			652		
Timely clinical data	52 58.4%			463 71.0%		
Timely lab data	53 59.6%			473 72.5%		

Follow-up forms due during the COVID-19 amnesty period from 3/13/2020-3/31/2021 are not included in timely clinical and lab data calculations